

Effective Leadership in a Health Service

A case study of a healthcare organisation that has recently undergone structural change

Alistair Squires

Declaration of originality

I, Alistair Squires, declare that I am the sole author of this dissertation, and that the work is a result of my own investigations, except where otherwise stated. All references have been duly cited.

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ABSTRACT

This research is about leadership. Not business leadership, about which literature abounds, but about leadership within the healthcare sector. Do people within healthcare see and define leadership any differently to others, or to general business literature? If so, does such leadership exist? This research aims to find the answers.

This report is about a case study in a large and geographically dispersed organisation that operates within an Australian public healthcare sector that had recently undergone significant structural change, and that required direction and leadership at a level not experienced before. The study sought to determine how healthcare leaders define effective leadership, and also whether or not there was evidence to suggest it exists in the organisation.

A sequential mixed-method of research was adopted, with ten semi-structured interviews and a questionnaire sample of 350 managers and leaders, being the primary sources of data.

Interviews were transcribed, then analysed in several cycles, the first of which provided input for development of the questionnaire, which was administered by using SurveyMonkey®. The interview transcripts and the questionnaire responses were analysed using both qualitative and quantitative techniques to develop the findings.

The data suggested that clinicians and non-clinicians define effective leadership similarly.

However, their assessment of its existence within the case-study organisation differed substantially, particularly in the area of behaviour, with clinicians seeing effective leadership where non-clinicians did not.

An internal definition of effective leadership is offered, and the overall conclusion reached is that effective leadership, *as defined and desired*, does not exist within the case-study organisation. However, the appropriateness of various leadership styles is discussed further.

Recommendations are offered that might assist the organisation to identify areas of weakness within individuals, teams, organisational units and the organisation as a whole, together with suggested improvement measures.

1.1 Background

Reform of the Australian health sector has been a key federal government strategy over the last few years. Fuelled by scandals and the resultant scrutiny, reform was particularly energised in Queensland by a change of state government in 2012 – see Appendix 1.

Queensland covers 1,852,642 square kilometres¹ (7½ times the area of the UK) with a widely dispersed population of 4.7 million². In this context, the central control of healthcare delivery was recognised as being problematic. In July 2012 Queensland was divided into 17 ‘districts’ for healthcare purposes (Appendix 2), with devolution of governance and management responsibility.

In each district, independent authorities, Hospital and Health Services (HHS) were established, with independent Boards of Directors and ‘corporate-style’ organisational structures. They are required to deliver healthcare services in terms of Service Agreements with the government’s Department of Health³ (DoH). HHSs are mostly funded through the state budget, notwithstanding an increasing emphasis on the generation of ‘own-source’ revenue.

The new HHS’s are free to adopt DoH policies, procedures and frameworks, but are encouraged to either ‘localise’ them, or to develop their own. It is expected that, over time, the HHS governance and leadership structures will take on a character of their own.

¹ Source: Wikipedia en.wikipedia.org/wiki/Queensland.

² Source: Queensland Government website www.oesr.qld.gov.au/products/briefs/pop-growth-qld/qld-pop-counter.php.

³ Source: AnHHS. Service Agreement 2013/14 – 2015/16. State of Queensland (Queensland Health), November 2013.

1.2 Target of the research project

My research targets one of Queensland's largest HHS's, which is identified as **AnHHS** herein.

AnHHS serves a population of $\pm 300,000$ in an area exceeding 90,000 square kilometres (over $\frac{1}{3}$ of the area of the UK), through 26 hospitals and clinics with $\pm 4,700$ staff.⁴

The executive management team comprises professionals, most of whom held senior management positions before the reorganisation, although their current roles may not necessarily be the same.

The Board of Directors is made up of prominent figures from different areas within the district and includes doctors, politicians and business people. Upon appointment, many members had no previous directorate or corporate governance experience. Key considerations in their selection included geographic location, community profile, business skills and experience.

1.3 Research question and objectives

Given the 'products' within the healthcare sector, health and life itself, is it reasonable to assume that general leadership and management principles apply? Is effective leadership within such organisations different to that found elsewhere? These questions require an understanding of the meaning of '*effective*' in the context of healthcare, as well as matters related to established leadership theory.

⁴ Source: AnHHS Annual Report 2012-2013

Whilst AnHHS, in its current form is a new entity, the available staffing and operational skills have existed for a long time. However, the localised ‘ownership’ of the *responsibility* for delivering public healthcare services is new. Effective leadership is needed in this new era, and this research seeks to establish if AnHHS has what Collins (2006:13) describes as ‘true leadership [that] only exists if people follow when they have the freedom not to.’

The specific research question, therefore, is:

“What makes leadership effective within a Health Service?”

To answer the question, the research aims to:

- a) gain an understanding of what the literature says about *effective* leadership in general, and in the healthcare sector in particular;
- b) establish the meaning and general understanding of *effective* leadership within AnHHS;
- c) determine whether *effective* leadership exists at AnHHS, as measured against theoretical and internal definitions;
- d) recommend strategies for improvement where appropriate.

1.4 Why do this research?

Current leadership literature presents various models of leadership characteristics, attributes, styles, skills and behaviours that may be flexed according to particular situations or contexts (Yukl, 2013), in order that leadership, as a body of knowledge can be generally applied within organisations. What is not clear, however, is whether this knowledge is entirely applicable in the healthcare sector, or whether there are other elements of leadership that are peculiar to healthcare.

A secondary reason for the research is that AnHHS is my employer, and my role is that of internal auditor and the potential for benefit to the organisation was identified. My hope is that the project will find traction amongst the leadership with a view to organisational improvement.

1.5 Scope, methodology and structure

The research focuses on leadership within AnHHS at senior management and Board levels and is considered from the epistemological position, employing an interpretivist philosophy and inductive reasoning approach.

Chapter 2 contains a detailed literature review that provides an understanding of the theoretical position on effective leadership, which forms the basis for implementing the research methodology.

Chapter 3 describes the case study strategy used, and the sequential multi-method approach that employed semi-structured interviews and survey questionnaires as the primary data-gathering instruments. These were exercised amongst various tiers of leadership within AnHHS in order to obtain an internal understanding of leader effectiveness, and whether there is evidence to suggest it exists in AnHHS.

Chapter 4 presents the findings from data obtained by the chosen methods, which are then analysed, discussed and interpreted with reference to the literature and theory introduced in Chapter 2.

Chapter 5 presents conclusions derived from the findings of the research, together with recommendations arising for the leaders of AnHHS to consider. The limitations of this research, together with some suggested areas for further research are also identified in this chapter.

Finally, Chapter 6 provides some reflection on the dissertation process and its impact on my personal development.

2.1 Introduction

The purpose of this chapter is to address the first research objective, which is to review the literature to understand what 'effective leadership' might mean and whether there are any matters peculiar to healthcare.

2.2 Literature search

Sources considered for this review included reputable books on leadership, as well as contemporary literature derived from various electronic sources such as university libraries (eg. YSJ – York St John University; QUT – Queensland University of Technology), databases such as McKinsey and Harvard, journals (both academic and non-academic), research articles and papers. Topical searches via Google revealed other sources of information (Fisher, 2010).

Given the confusion that often arises around the difference between leadership and management, literature on this matter is referenced, as is the distinction between various 'streams' of leadership within the health sector, such as Board, Executive and Clinical leadership. It also became evident that the published experiences of the National Health Service in England would be relevant to this study, particularly in the context of the Queensland healthcare sector and AnHHS.

2.3 Literature review

2.3.1 Distinction between leadership and management

In 1975 Zaleznik & Kets de Vries made one of the earliest distinctions between managing and leading (Conger, 1999), before which leadership and management were mostly viewed as synonymous. They argued that the inability of American corporations to adapt to change was due to too much management and not enough leadership.

Management is considered to be practical and all about ‘doing’ things, whereas leadership brings about constructive or adaptive change – ‘managers do things right and leaders do the right things’ (Bennis, 1997). Another distinction between leadership and management (Table 1) indicates that management does the day-to-day things for organisational function, but leadership is a process of enacting ideas that create change.

Table 1: Distinction Between Management and Leadership

Source: Adapted from Kotter (1990)

Management	Leadership
Is about: • Consistency • Predictability • Order	Is about: • Producing movement • Organisational change
Comprises these processes: • Planning and budgeting • Organising and staffing • Controlling and problem-solving	Comprises these processes: • Establishing direction • Aligning people • Motivating and inspiring people

It seems convenient to theorise about leadership and management, and that neither can exist without the other but what about the person? Can a person be both leader and manager at the same time? Andersen (2012) suggests a person can do both, so a leader leads and manages, and vice-versa. Each will demonstrate management and leadership, as circumstances and situations require (Yukl, 2013).

2.3.2 A brief look at the evolution of leadership theory and models

Throughout its evolution, leadership literature, written mostly for business, is littered with words such as traits, attributes, skills, behaviour, transactional, transformational, styles, qualities, emotional intelligence, self-awareness and so on (Bass, 2008; Gentry et al, 2012; Gill, 2011; Senior and Swailes, 2010; Yukl, 2013), accompanied by a multitude of models.

In the last 100 years, this evolution has seen the concepts, theories and models go through several phases. Initially, trait theories supposed that leaders were *born* with certain leadership characteristics or attributes (Gill, 2011), and you either had them or you didn't. This evolved into the notion that leaders could be *made* (Yukl, 2013), which required the identification of various skills and knowledge that could be learned (Northouse, 2013; Yukl, 2013).

Further evolution led to the consideration of what leaders do and how they act i.e. their behaviours and styles. The famous Michigan and Ohio studies, led to the development of measurement tools such as the Leader Behaviour Development Questionnaire (LBDQ) and Blake & Mouton's Leadership Grid (Northouse, 2013; Senior & Swailes, 2010).

Along the way, it became obvious that there was no 'one-size-fits-all' type of leadership, and that certain situations demanded different leadership approaches, where context can affect leadership processes (Yukl, 2013) – contingency, and situational theory evolved.

In considering these matters, a distinction was drawn between leaders who are Transactional and those who are Transformational (Downton, 1973), later popularised by Burns (1978) in his research on political leaders, and then by Bass (2008) who introduced it to organisational psychology. Together with his colleagues, Avolio et al (2003), Bass expanded his research and

identified the 'Full Range Leadership Theory' (Table 2), with components of transformational, transactional and non-transactional leadership, later developing his Multifactor Leadership Questionnaire (MLQ) for gauging leadership qualities.

Table 2: Summary of Full Range Leadership Theory

Source: Adapted from Avolio et al (2003:264-5)

Style	Components	Description
Transformational	Idealised influence – attributed	refers to perceptions of the leader
	Idealised influence – behaviour	referring to the leader's actions centred on values, beliefs and mission
	Inspirational motivation	the way leaders energise their followers
	Intellectual stimulation	appealing to the followers' sense of logic and analysis, and creating challenge for the followers, encouraging them to be innovative, creative and inquisitive
	Individualised consideration	paying attention to followers' needs
Transactional	Contingent reward	based on reward, or consequence for performance or otherwise and is considered a 'constructive' transaction, i.e. in order to get something done
	Management by exception – active	monitoring of performance and taking corrective action as necessary
	Management by exception – passive	the leader deals with things if and when they come to his attention
Non-transactional	Laissez-faire	'what will be will be'

Arguably the most important element of leadership to arise in this evolution is the concept of Emotional Intelligence, commonly referred to as EI or EQ (much as Intelligence Quotient is known as IQ), which received attention in the mid-1990's and is defined as 'the ability to perceive and express emotion, assimilate emotion in thought, understand and reason with emotion, and regulate emotion in the self and others.' Mayer et al (2000:396).

Goleman (1998) developed an Emotional Intelligence model (Table 3), highlighting four fundamental capabilities that summarise a range of characteristics. This list resembles other lists of attributes, which implies that one who is truly Emotionally Intelligent can sense circumstance and discern the appropriate style, skill and behaviour to apply. In further work on the subject, Goleman (2000) linked EI competencies to a range of leadership styles (Table 4) that would be appropriate in certain circumstances. Again, the capacity to sense and apply the appropriate style comes with Emotional Intelligence.

Table 3: Summary of Daniel Goleman's Emotional Intelligence Model

Source: Adapted from Goleman, 1998

Self-Awareness	Self-Management	Social Awareness	Social Skill
Emotional Self Awareness – knows own emotions and their impact on others	Self-control – Controls emotions and impulses	Empathy – senses others emotions, understands perspectives, interested in others	Visionary leader – takes charge, inspires with vision
Accurate self-assessment – knows strengths and weaknesses	Trustworthy – displays honesty & integrity	Organisational awareness – savvy, and able to navigate organisational politics	Influences – persuades
Self-confident	Conscientious – manage self and responsibilities	Service orientation – identifies and meet needs	Develops others – provides feedback and guidance
	Adaptable – deals with changing situations and obstacles		Communicator – good listener & sends well-tuned messages
	Achiever		Change-catalyst – new ideas, new direction
	Initiative – seizes opportunities		Conflict management – de-escalates strife
			Builds bonds & relationships
			Teamwork and collaboration

Table 4: Linkages between Leadership Styles and Emotional Intelligence Competencies

Source: Squires, 2013; Goleman, 2000

	Coercive	Authoritative	Affiliative	Democratic	Pacesetting	Coaching
Leader's Modus Operandi	Demands immediate compliance	Mobilises people towards a vision	Creates harmony and builds emotional bonds	Forges consensus through participation	Sets high standards for performance	Develops people for the future
Style in a phrase	"Do what I tell you"	"Come with me"	"People come first"	"What do you think?"	"Do as I do, now"	"Try this"
Underlying EI competency	Drive to achieve, initiative, self-control	Self-confidence, empathy, change catalyst	Empathy, building relationships, communication	Collaboration, team leadership, communication	Conscientious, drive to achieve, initiative	Developing others, empathy, self-awareness
When the style works best	In a crisis, to kick-start a turnaround, problem employees	When changes require a new vision, or when a clear direction is needed	To heal rifts in a team or to motivate people during stressful circumstances	To build buy-in or consensus, to get input from valuable employees	To get quick results from a highly motivated and competent team	To help an employee improve performance or develop long-term strengths
Overall impact on climate	Negative	Most strongly positive	Positive	Positive	Negative	Positive

This review will not delve any further into the history, as it seems that there is growing agreement on aspects such as attributes, skills and behaviours, although presentation and words may differ. More focus will be placed on those aspects that might point to effectiveness of leaders, and whether there are differences within healthcare.

2.3.3 Levels of leadership in AnHHS

As this research is in the context of a healthcare organisation, a brief distinction will be made between the leadership of Boards, Executives, Clinicians and other levels. It is anticipated that this distinction will be encountered in later research to be conducted.

Board

The Board of Directors leads the overall organisation in terms of strategy, accountability, culture, contextual understanding and intelligence, engagement of all stakeholders, vision and direction (NLC, 2010a; Garratt, 2010).

Executive

The Executive leads the organisation from an operational perspective, implementing the strategies and vision of the Board, but measuring their success in terms of financial metrics, operational statistics and budgets.

Clinical

The 'clinical' component of leadership commonly refers to medical, nursing, allied health and paramedical staff i.e. anyone with a clinical patient role, and has been variously defined as:

“...leadership of clinicians by clinicians...”;

“...an expert clinician...who influences others to improve the care they provide...”;

“...is the capacity to drive the vision of delivering safe health care...” (Garurubba et al, 2011).;

“...clinical leadership occurs at all levels of patient care, and refers to leading activities that improve the delivery of safe clinical care, and the set of attributes required to lead a team, unit, stream or cluster.” (CEC, 2014)

Table 5: NHS Leadership Qualities Framework

Source: NHS (2006)

Personal Qualities	• Self belief • Self awareness • Self management • Drive for improvement • Personal integrity
Setting Direction	• Seizing the future • Intellectual flexibility • Broad scanning • Political astuteness • Drive for results
Delivering the Service	• Leading change through people • Holding to account • Empowering others • Effective and strategic influencing • Collaborative working

The National Health Service in the UK developed a leadership framework (Table 5), the outcome of intense study and research within a proactive national healthcare sector, and provides good insight to sector-specific qualities. Not surprisingly, this schedule could easily apply to any non-healthcare organisation as well.

Other

Leadership will also be seen in a healthcare organisation at various other functional levels such as Finance and Administration, Facilities, Human Resources etc., much like any other business.

2.3.4 What does effective leadership look like?

Many qualities deemed essential for leadership are also desirable in peers as well as followers.

Any leader wants followers who are honest, hardworking, respectful, trustworthy etc. (Gill,

2011), so these can certainly not be viewed as the domain of leaders only. They should be considered as core attributes of any *ideal* employee, particularly in healthcare, where staff are dealing with health and life needs (CCL, 2010).

For leaders to lead, there needs to be followers. Organised collections of individual followers form teams. Effective teams need effective leaders. In the context of AnHHS, where significant organisational restructuring has taken place, the forming, storming, norming and performing stages of Tuckman's (1965) model of team formation should be recognised and expected. This will more than likely call for the employment of Hersey and Blanchard's (2012) situational leadership model where the stages of telling, selling, participating and delegating will be applicable, as individuals and teams progress through various levels of maturity. Effective leaders should recognise their role in the various stages of team development (Table 6).

Table 6: Stages in Team Formation

Source: Adapted from Tuckman (1965) and Hersey et al (2012)

Tuckman	Hersey & Blanchard	Leader role
Forming	Telling	Introduce and direct
Storming	Selling	Build trust, develop relationships, resolve conflicts
Norming	Participating	Settle and build team
Performing	Delegating	Delegate, do

Illustrating just how theories of leadership and management evolve, and can do so in a relatively short space of time, Senge (2006:x) quotes Dr W.E. Deming, an early pioneer of the concept of Total Quality Management (TQM), who has had a change of heart about certain aspects of his previous thinking. Deming now says that 'Our prevailing system of management (including TQM) has destroyed our people', and rather that 'People are born with intrinsic motivation, self-respect, dignity, curiosity to learn, joy in learning.' These comments align with Senge's 'three-legged stool' model (Figure 1) for learning teams, in which he presents his five learning disciplines of personal mastery, shared vision, mental models, dialogue and systems thinking, as

comprising the three learning capabilities: fostering aspiration, developing reflective conversation and understanding complexity. Effective leadership seems to be connected to bringing these intrinsic qualities out of people, for application in the workplace as 'the critical moment comes when people realise that this learning organisation work is about each one of us.' (Senge, 2006:xvi).

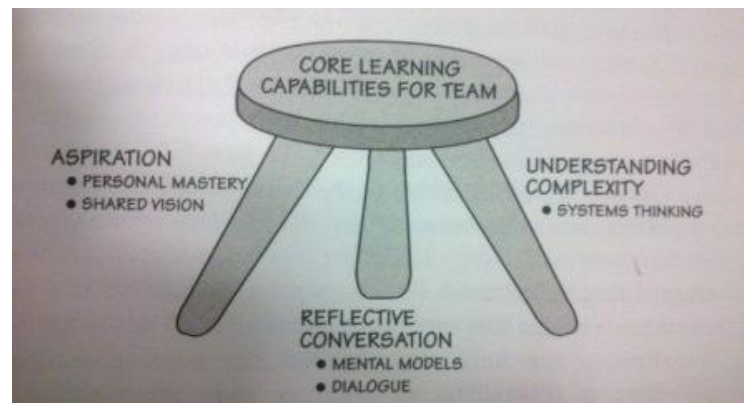


Figure 1: The three-legged stool of learning teams

Source: The Fifth Discipline (Senge, 2006)

In her review, Marrow (2010) cites the research of Hannah and Lester (2009) into learning within complex organisations – AnHHS would fit this description. They proposed a multi-level model of learning, suggesting that effective leaders can facilitate learning at the micro (individual), meso (social networks) and macro (organisation) levels by establishing the conditions and structures that provide appropriate challenges, opportunities and experiences within the workplace. This resonates with Senge's (2006) team-learning concept discussed earlier where 'micro' aligns with 'personal mastery', 'meso' with 'shared vision' and 'conversation', and 'macro' with 'systems thinking'.

Any leader is an individual, probably a member of some form of team, and will have experienced, or may still be actively engaged in, being a manager which has then resulted in him being the leader (Conger, 1999). This is not necessarily the progression, but Collins (2001) refers to a hierarchy, towards what he calls level 5 leadership, alluding to characteristics of the person as

simply an individual, a team member, a manager, or leader (Figure 2), although an effective leader at any level is expected to embody levels 1 to 4. An unexpected finding by Collins was that it seemed a waste of energy and time for leaders to motivate people, which assume of course that the right people are on board, in which case they will be *self-motivated*. The trick however, is not to *de-motivate* them.



Figure 2: Hierarchy of executive capabilities Source: Good to Great (Collins, 2011) www.unityasone.wordpress.com/2012/09/21/collins-level-5-hierarchy [image accessed 20 July 2014]

Many aspects of Collins' hierarchy can be identified and aligned with the findings of CCL's (2010) analysis of leadership competencies in the healthcare sector, using their Benchmarks[®] framework, a 360° assessment tool, which measures 16 competencies and 5 derailment factors (Appendix 3). Assessments from ±35,000 healthcare workers in the US (e.g. large hospitals, regional providers, federal healthcare agencies, pharmaceutical firms, medical device manufacturers), accumulated over 10 years (2000–2009), were ranked in terms of 'importance for success', and provide clues as to what might be expected in effective leaders in healthcare.

CCL's 'derailment factors', which include: difficulties in building good working *relationships* and in building and *leading teams*; resistance to *change, learning and development*; and lacking

breadth of orientation i.e. unable to manage outside one's own function, also provide pointers to effectiveness. Schein (2010) also asserts that this flexibility and openness to new ideas is key for leaders, to enable faith and trust in their people so they, in turn, can develop self-belief for learning, and freedom to experiment.

Again, it is not difficult to identify the formation and leading of teams as being fundamental to effective leadership within the healthcare sector. The difference between simply having a team, and having the *right team* is not lost on Richard Bracken (2013), CEO of Hospital Corporation of America (165 hospitals, 200,000 employees), who relates effectiveness to having the right team, with the right values, in place to execute the plan but can still 'pivot' when factors change. The leader's intellectual flexibility is also identified, which translates to *team* flexibility for the 'pivoting' that Bracken describes. This aligns directly with Collins' (2001:13) image of the 'right people on the bus, the wrong people off the bus, and the right people in the right seats'.

Leadership in teams is seen to be the ability to influence others toward the achievement of a particular goal, and as in sports teams, healthcare is composed of a roster of very talented and skilled professionals and employees, who must work together to deliver the goal of quality health services (Burke and Friedman, 2011). The crucial nature of effective communication of the leader's vision is again implied here, by the 'ability to influence'.

Although self-awareness only appears midway down the CCL (2010) ranking (Appendix 3), it was identified by Bennis (1997) as being key in effective leadership, alongside being 'grabbed by *vision*', conveying vision with meaningful and compelling *communication*, managing one's own *trustworthiness* as well as *trusting* others. Self-awareness can be seen as the recognition of personal strengths and weaknesses, self-belief, sufficient self-confidence to be open to new ideas, and continual learning (Burke and Friedman, 2011).

Whilst Collins (2001) grouped his ideas into the 5 level hierarchy, and CCL (2010) identified 16 skills, five practices were identified by Kouzes and Posner (2012) in their Exemplary Practices model (Table 7) as being common to all effective leaders. This commonality of approach indicates the ‘core-ness’, and therefore the essential nature of the practices. An in-depth look at these behaviours is considered unlikely to reveal anything that has not already been identified in another list or model, but would merely serve to reinforce their validity.

This need to be flexible in times of change, by innovating and learning, resonates with the key theme of Kotter’s (1990, 1996, 2002) writings, together with the sentiment expressed by Burger and Giger (2014) that as healthcare leaders cope with the uncertainty brought on by legislative changes, there is one constant they can anticipate: *change*. Clearly, the ability to strategise and cope through change is a key factor in effective leadership, whether in healthcare or business.

Table 7: Exemplary Practices Model

Source: Adapted from Kouzes and Posner (2012)

Model the Way	leading by example, setting standards of excellence, creating opportunities for others
Inspire a Shared Vision	visualising the future, enlisting others, sharing the vision
Challenge the Process	innovating and taking risks, continuing to learn
Enable Others to Act	involving others, respecting others, trusting and strengthening others
Encourage the Heart	recognising the contribution of others, celebrating organisational achievements

The creation of Kotter’s (1996) guiding coalition is the underlying concept for Wilderspin’s (2013) view that the transformation of healthcare requires effective leadership to bring together the ‘key movers and shakers’ of local governments, NHS (National Health Service, UK), CCG’s (Clinical Commissioning Groups) and provider organisations. This would probably be evident at a Board rather than Executive level, but nevertheless forms part of the landscape of effective leadership.

A useful comparison of leadership in the business sector and social sectors (Appendix 4), which includes healthcare, was carried out by Collins (2006), and he concludes that effectiveness is not a function of circumstance, but of conscious choice and the discipline to face varying difficulties and constraints. He suggests that the relative advantages and disadvantages of each sector more or less cancel each other out. Having said that, this comparison is quite revealing in terms of the differences in circumstances faced by business and healthcare organisations. Leaders in each will have to draw upon their repertoire of abilities. (Simmons & Fraser, 2012; Kings Fund, 2013).

2.4 Conclusion

There seems to be no limit to the number of words, ideas and models about what effective leadership means and might look like in reality. It would be difficult to identify any 'best' collection of qualities without creating yet another list. It has not become clear from the literature whether there are any leadership factors that are unique or peculiar to healthcare, nor that some may be found in other sectors and not in healthcare.

What has become clear is that leadership is essential, organisational success and achievement is dependent on effective teams, and effective teams are effectively led. It stands to reason then, that effectiveness is a result, which is dependent on many factors over which a leader has great influence. Can it be said that an organisation's success or failure is therefore dependent on the leader, or can it be achieved *despite* the leader?

I suspect that the last book about these matters has not yet been written, and wonder about de Vries' (1994) comment of 20 years ago that 'as far as leadership studies go, it seems that more and more has been studied about less and less, to end up with a group of researchers studying everything about nothing'.

Certain themes and concepts identified in the literature (e.g. attributes, skills and behaviours) were used in the development of the interview schedule and questionnaire for later application in primary research within AnHHS, and also arose in the subsequent qualitative analysis.

3.1 Introduction

The purposes of this chapter are to provide a rationale for the research, to explain the philosophy, approach and design of the research, to consider the access to data sources, to address ethical issues surrounding the research, and to describe how the research was carried out to meet its objectives, namely to obtain an internal understanding of leadership within AnHHS, and an internal evaluation thereof.

3.2 Rationale for this research

This research concerns leadership, particularly within the healthcare sector, but drawing heavily on concepts and knowledge in the business world. The subject was chosen for various reasons including the circumstances of my employer organisation, my interest in the subject, my access to a variety of sources of primary and secondary data, the availability of literature, my anticipation of meaningful results, and, of course, the relevance of the topic to the MALIC programme requirement.

3.3 Research philosophy and approach

Leadership, as noted by many writers, is difficult to define absolutely, and is affected by situations that influence outcomes and leader behaviour (Yukl, 2013). The Research Onion model (Figure 3) guided my approach to this research and I took the epistemological view that I would be researching the 'nature of knowledge', and that the subject matter did not lend itself to the factual and statistical research such as that found through ontological studies of reality (Fisher, 2010).

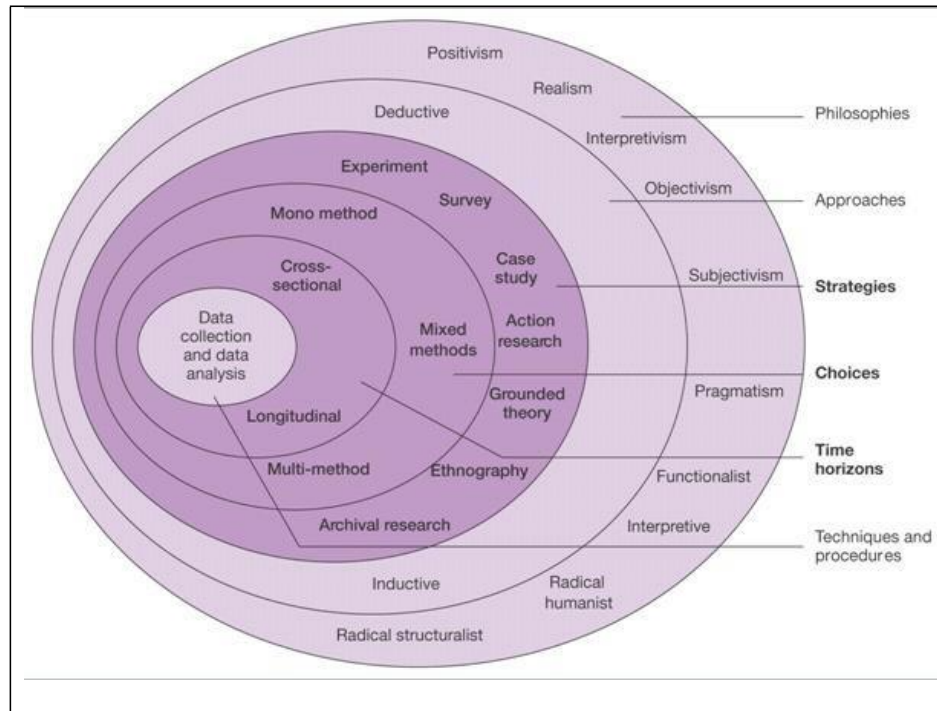


Figure 3: The Research Onion Model (adapted from Saunders et al 2012)
Source of image: <http://writepass.com/journal/2013/04/sample-dissertation-methodology/> [Accessed 25 May 2014]

Philosophy

Given this epistemological stand, and the intended consideration of views, interpretations, perspectives and opinions about the way AnHHS should operate its affairs; an interpretivist research philosophy has been employed. This ‘feelings’ philosophy calls for an understanding of differences between humans ‘on the stage of life’ (Burrell and Morgan, 1982), and is considered more appropriate than other philosophies illustrated in Figure 3.

Approach

As I would be exploring the phenomenon of healthcare leadership to identify factors pointing to *effective* leadership, and allowing meaning to emerge from data gathered through the interviews

and questionnaire, an inductive reasoning approach was adopted, unlike the deductive approach that identifies a theory that is then tested to draw meaning (Saunders et al, 2012; Fisher, 2010).

3.4 Research design

3.4.1 Strategy

How would I find out the answer to the research question without looking at a real life situation that would facilitate inductive and exploratory research (Ghauri, 2010)? Did I have access to such a situation, and if so, would it be permitted? I work for AnHHS, and approval was a real prospect, so I selected AnHHS as the 'unit of analysis' in a case study (Yin, 2014). This was ideal, as my role permitted access to multiple sources of data, which would provide the 'triangulation' necessary for valid conclusions (Robson, 2002).

3.4.2 Methods

Narrative arising from documents, interviews and conversations (qualitative, 'wordy' techniques), as well as statistical information derived from a survey questionnaire (quantitative, 'numbers-type' techniques), provided the data for this research. The generally accepted view that qualitative methods are useful for inductive research, especially at the first stage, applied in this instance, as it allowed face-to-face and exploratory enquiry. This was followed by the second stage, quantitative method of the survey questionnaire, which tested some of the findings of the interviews (Ghauri, 2010). Whilst described separately as qualitative and quantitative techniques, combinations of the two were also employed (e.g. the frequency of occurrence of words), which fits Saunders' (2012) description of a complex mixed method. Some of the phases

followed one another, further characterising it as sequential mixed-method research (Creswell and Clark (2007), cited by Saunders 2012:167).

My position as Head of Internal Audit within AnHHS facilitated a collaborative combination of the 'engaged' and 'participant' research models, enabling regular contact with the very leaders that formed the groups under consideration. Although not employed throughout the project due to its limited application, I was able to obtain the views and opinions of a range of people with different perspectives, in keeping with the interpretivist philosophy behind the research (Fisher, 2010).

3.4.3 Data collection instruments

The specific primary research instruments used were semi-structured interviews and a questionnaire. Whilst consideration was given to using secondary research sources such as internal reports and surveys, I felt that this would serve little purpose other than to provide evidence of organisational achievements and results. As this was not a focus of the research, I chose to limit my research to primary data, which was expected to be rich enough.

No method was considered better than another as they each served a different purpose. Interviews were selected for their capacity to obtain direct input, with the opportunity to clarify, understand, explore and probe. A questionnaire was used to get a wider view on targeted matters, expecting that a more generalised perspective would be obtained (Ghauri, 2010). There was ready opportunity to employ these methods due to access, geographic proximity of the sources and the relative ease with which they could be arranged.

Other primary research instruments were considered and rejected for various reasons. The use of panels and focus groups was rejected due to the geographic spread of potential participants and the time constraints attached to the research. Observational research was seriously considered, and even commenced briefly, but its invalidity, and subsequent rejection, was recognised when it became obvious that I would not be able to meaningfully observe more than a handful of AnHHS' leaders.

This consideration, and use of a range of data collection methods, aimed to provide reasonable assurance that information derived from one source of evidence was consistent with others – the process of triangulation (Saunders et al, 2012), but also to ensure I had the opportunity to engage with individuals as well as obtain the views of as wide an audience as possible.

Being conducted once and over a limited period of time, this research should be viewed as cross-sectional, although the subject of leadership within AnHHS is an ongoing consideration and therefore lends itself to ongoing, longitudinal research over a longer period.

3.5 Project approval, access and support

Having decided to target AnHHS, for the case study, formal approval of the research was sought from the Chief Executive and the Chair of the Board Audit and Risk Committee. Support was freely given, together with advice on various internal steps that needed to be taken, such as the ethics approval process.

Not wishing to abuse my uniquely privileged position of internal auditor as regards access to staff, information and records, approval to use this access for research purposes was also sought, and readily given, opening the way for meaningful selection of research material.

In addition to project approval and ready access, AnHHS lent its support in the form of time allowances for study and research purposes, which proved invaluable.

3.6 Ethical considerations

As the research project was conducted within the healthcare environment, the ethics issue was a key consideration. The *matter* was raised very early in the process when approval for the project was being sought, at which time I became aware of the strict processes surrounding any form of research within healthcare.

As the project hinged on this approval, an early submission was made to the Human Research and Ethics Committee (HREC), a statutory body with administrative representation in many institutions around the state, including healthcare. Although the formal academic proposal had not yet been submitted, the nature of the HREC application form was such that broad ideas and intentions were adequate. Approval was obtained (Appendix 5), on condition that I kept the Committee informed of the project's progress, and to obtain specific approval for certain aspects such as the administration of the questionnaire. This was duly obtained (Appendix 6).

The 'informed consent' recommended by Fisher (2010) was attained by ensuring that all participants, interviewees and questionnaire respondents alike, were made aware of the purpose of the research, their freedom not to participate and that all data would be treated anonymously and confidentially. This is discussed further when detailing the application of the research methods below.

3.7 Primary research

As discussed earlier, a sequential mixed-method strategy was employed, using interviews and a questionnaire.

3.7.1 Interviews

The logical first step was, in my mind, to discuss these things with some of the organisational leaders themselves, to be drawn from the Board of Directors and the Senior Executive team.

Selection process

I decided to conduct ten interviews, 5 from the 10 Directors and 5 from the 7 Executives. The participants were selected purposively (Fisher, 2010) to meet my criteria (Table 8), to ensure a variety of skill, background and experience.

Table 8: Criteria for Participant Selection

Criteria	Detail
Sample size	10
Category	5 Directors and 5 Executives
Clinically trained	As close to 50:50 as possible
Clinically active	No target ratio, but required good representation from both active and non-active
Non-health experience	As close to 50:50 as possible. Hoped for some that had never worked in non-health environment, as well as some that had extensive experience outside the health sector
Gender	Not specified

I researched the backgrounds of all Directors and Executives by studying their profiles (Appendix 7), which were available on the AnHHS Intranet, conducting general research on the Internet, from private conversations with trusted personalities within AnHHS, and from my own

first-hand knowledge and observations of the candidates. The participants selected for interview included a former parliamentary Minister of Health, senior medical doctors and specialists, former and current Chief Executives from health and other backgrounds, and financial experts and administrators. Whilst there was no intention to do so, there was a gender imbalance in the selection, although this was considered inconsequential to the research (Table 9).

Table 9: Analysis of Interview Participants

Candidates	Category		Gender		Clinically trained		Clinically active		Non-health sector experience	
	Director	Executive	M	F	Yes	No	Yes	No	Yes	No
Interviewed										
1		✓	✓			✓		✓	✓	
2		✓	✓		✓			✓	✓	
3		✓		✓	✓			✓		✓
4	✓			✓	✓		✓			✓
5	✓		✓			✓		✓	✓	
6	✓		✓		✓		✓			✓
7	✓		✓		✓		✓			✓
8		✓	✓		✓			✓		✓
9	✓		✓			✓		✓	✓	
10		✓	✓			✓		✓	✓	
Total	5	5	8	2	6	4	3	7	5	5

Interview documentation

An information sheet was prepared, which was provided to interviewees prior to the interview (Appendix 8). This sheet, designed to establish the ‘terms of engagement’ (Fisher, 2010), included an early notice of my request to make an audio recording of the interview (discussed later), and also called for a signed consent to the process (Saunders et al, 2012:239).

I also prepared an interview schedule (Appendix 9) for my use during the interviews, which drew on matters that arose in the literature review, together with my previous knowledge and observations (Wilson, 2006). Questions were designed to direct the interviews in a way that would help me to gain insights to what leadership should consist of within AnHHS, as well as to

get a sense of what the corporate leaders themselves thought about the effectiveness of leadership within AnHHS.

The interviews were designed as semi-structured, starting with obtaining an understanding of the interviewee's background and leadership journey, then moving on to their views on leader qualities considered essential, and finally their assessment of the effectiveness of current leadership within AnHHS. All interviews were conducted face-to-face for a number of reasons, including: geographic proximity, inconvenience of long phone calls to busy participants, body language and facial expression being lost when using telephone interviews as well as the fact that I could not ensure privacy and confidentiality at the participant's end of the telephone call, and so on. For consistency, all interviews were conducted on the same basis, using the same schedule and conversational style, over a relatively short period of time (Bryman, 2011).

Pilot interview

Before sending out the information sheet, or arranging interviews, I did a 'test-run' of the interview (Wilson, 2006) with a colleague who is a qualified health practitioner and had also done research at a Masters degree level. I provided him with the pre-interview information sheet and sent him a mock invitation email. I recorded the interview, which allowed me to test the reliability, ease of use and the sound quality of the device that I had purchased, a Sony ICD-AX412F.

I used the interview schedule and conducted the interview diligently, which took 40 minutes. This process allowed me to check the understanding of the interviewee regarding the research problem and interview questions. It also enabled me to estimate the time expected for the

interviews, which would be a useful indication when inviting interviewees to participate (Ghauri, 2010).

Feedback from my colleague was helpful and resulted in certain changes to preliminary information as well as the clarification of objectives and questions.

The interviews

Appointments with interviewees were made in person or by telephone where necessary. Emails confirming these arrangements were then sent to participants with the above-mentioned information sheet attached (Appendix 8), reminding them again that confidentiality and anonymity were assured, as well as the request to make an audio recording of the interviews.

I wanted to make interviewees feel comfortable and tried to conduct the interviews more as *naturalistic* conversations, than rigid interviews (Wilson, 2006). In some instances, participants responded to questions and delivered valuable data. In other instances, they needed more 'hand-holding', in which case I was mindful of the risk of stepping over the line and imposing my own beliefs and interpretations – *interviewer bias*. In some situations, it was difficult for the participant to see me as a researcher, and not the internal auditor, with the associated expectations – sometimes described as *participation bias*. It may be that some interviewees made comments with a bias towards my internal position as opposed to their own views. This is sometimes called *interviewee or response bias* and, I believe, worked to my advantage in that the integrity, confidentiality and independence associated with my internal position allowed interviewees a level of freedom to discuss more candidly, allowing me to gain insights that might not otherwise have been possible (Saunders, 2012; Ghauri, 2010; Robson, 2002).

Audio recording of the interviews

The options for capturing the data were for me to listen and take notes as best I could, and to add the 'failsafe' recording to the process. I chose to do a digital recording for a number of reasons, all of which were realised: it took away my anxiety about 'missing something important', especially the verbatim quotes that I was hoping to capture for analysis; it allowed me to have eye contact with the interviewee and observe facial expressions, which were invaluable, enabling me to detect whether I could probe further or not; after an initial period of self-consciousness, it made the interviewee more comfortable as there was no frantic note-taking which could have given the impression that some things being said were more important than others; I could listen better and engage more with the interviewee; and it released me to make notes on other contextual matters such as interruptions, attitude and so on. On a few occasions I was asked to turn off the recorder, more for interruptions than for what was being disclosed. Another consideration was that if I only took notes, I would have to 'reconstruct' and transcribe the interview from memory, which in my mind would reduce the accuracy and reliability of the work (Saunders, 2012; Fisher, 2010).

The only disadvantage I felt may have weighed against my recording the interviews, was the possibility of interviewee discomfort and fear that something incriminating might be said. To mitigate this, I tried to provide convincing assurances of anonymity and confidentiality. There were no objections to recording, and signed authority to do so was obtained from all interviewees (Saunders, 2012).

Transcription of the interviews

Primarily as a timesaving tactic, and with the full knowledge of interviewees, the digital files of recordings were uploaded to an online provider of transcription services, www.rev.com, and transcribed at a fee, based on recording time. Generally, raw transcripts were received within 24–48 hours and an average of 2 hours was spent on ‘cleansing’ each transcript, which provided an opportunity to obtain a ‘first look and listen’, and to gather my thoughts. The cleansing entailed correction of any transcription errors, standardising the formatting and inserting line numbers and time marks to facilitate later analysis. Although offered to all participants as a check for accuracy, validity and credibility for later findings, only one requested a copy of the transcript (Bryman, 2011; Saunders, 2012).

Coding of the transcripts

A first cycle of coding (see Appendix 10) was conducted by listening to the recorded interviews at the same time as marking the transcript, the purpose being to identify key words and their ‘frequency of occurrence’ – a content analysis technique (Easterby-Smith, 2012). This multitude of key words was further coded into the key themes of attributes, skills and knowledge, and behaviours; themes that were also seen in the literature. This information was later used to develop the structure and content of the survey questionnaire.

The interviews were then reviewed twice more, first by reading and audio together, then by reading only, resulting in the identification and organisation of a number of key thoughts and concepts (Appendix 11)(Coffey & Atkinson, 1996). A set of themed ‘quotable quotes’ from the interviews (transcripts identified by T1, T2 etc) can be seen at Appendix 12.

3.7.2 Questionnaire

This research journey began with the idea that the effectiveness of leadership would be explored from the various perspectives of middle management, executives and Directors, and that level-appropriate questionnaires would be developed for each. From the interviews alone, it became clear that this would be far more than this research paper could cope with, in light of both time and word-count constraints, and that it was likely that the data captured from a single questionnaire would be rich in its own right. The questionnaire did not, therefore, explore Board leadership, a subject that can be considered for further research.

Questionnaire design

The questionnaire had to be designed (in terms of question structure, options for response etc) in a way that met the research objectives to obtain an understanding of the internal views of leadership and what it ***should*** look like, and an evaluation of what a significant proportion of the organisation's leaders and managers thought about how ***actual*** leadership measured up to these definitions (Oppenheim, 1992).

The single questionnaire was prepared using information drawn from the literature review as well as the first cycle of analysis of the interviews (Williams, 2003). The questionnaire was divided into four sections as follows:

- SECTION A – Respondent information;
- SECTION B – What leadership ***SHOULD BE***;
- SECTION C – An evaluation of what leadership currently ***IS***; and
- SECTION D – A ***SELF***-evaluation.

Each of sections B to D contained exactly the same questions exploring the three areas of attributes, skills and behaviours, but from three different perspectives: section B to provide an internal definition of effective leadership; section C an evaluation of current leadership; and section D an opportunity for self-reflection. Section A data permitted cross tabulation with data, such as views of clinicians versus non-clinicians. I could have, but did not feel it necessary for purposes of this research, cross-tabulated the data to obtain separate views according to gender, age, length of experience and so on.

I also hoped that section D would provide an internal triangulation, or validation, as the respondents made up a substantial proportion of the actual leadership in the organisation, and there was some expectation that the overall evaluation of others would equate to the overall self-evaluation, being a 'closed loop' of evaluat-ees and evaluat-ors.

The questions were worded simply, avoiding jargon and difficult words. Closed questions were asked, requiring selection of a single answer from a 5-point Likert scale, framed as ranging from 'Very important' to 'Not important at all' in section B, and in sections C and D from 'Strongly agree' to 'Strongly disagree' (Oppenheim, 1992). This approach was chosen for a number of reasons, including the reduction of time and effort by the respondent, uniformity of questions to all respondents and ease of later analysis (Miller, 2002). The sections and questions were all provided with explanatory text to provide context and clarity. Most questions provided a box for comments, although none was called for or expected.

There are different schools of thought as to the order of questions, some suggesting that simple demographic questions such as age and gender be at the end of the questionnaire (Saunders et al, 2012), by which time the respondent would have provided most of the sought information. However, my personal preference when responding to questionnaires, which is the approach

that I selected, is to start with the simple matters and allow momentum to build for responding to the ‘meaty stuff’, a view shared by Miller, 2002.

Pre-piloting and piloting

As I was a sole researcher, time and expense was a factor in determining how the questionnaire would be administered. This, together with the fact that it was key for me to provide anonymity to respondents in order to increase the rate and quality of responses (Miller, 2002), led me to decide that respondents would complete the questionnaire themselves as opposed to the questionnaire being used within a structured interview setting.

Accordingly, I captured the questionnaire into an online survey tool, www.surveymonkey.com and pre-piloted it with my wife, for user-friendliness, clarity and ease of understanding (Williams, 2003). Her feedback was invaluable as it identified areas that could cause confusion, such as seeing the same questions three times without being warned to expect this as part of the structure of the questionnaire. These concerns could not be addressed directly with an online respondent, as might have been the case in a face-to-face administration of the questionnaire. As a result, the introduction, questionnaire overview and instructions were expanded. The amended questionnaire was then formally piloted with three senior colleagues, who would also be amongst the eventual respondents. Again, useful feedback was received and incorporated into the final questionnaire on SurveyMonkey® (Appendix 13).

Ethical approval

The conditions of the Ethics Committee’s approval for the research project required me to seek approval for the administration of any survey, which was duly obtained (Appendix 6).

Sampling process

To ensure that the data obtained from the questionnaire would be considered reliable, I required a confidence level of 95% with a 5% margin of error, and established that the necessary sample size should be 357, according to tables relevant to small, known populations (Appendix 14), given that the staff population of AnHHS is $\pm 4,700$.

The 'Manager' group on the email distribution list maintained by the Human Resources department of AnHHS was obtained and consisted of 372 email addresses, a sample much greater than the 100 anticipated at the research proposal stage, although no more difficult to administer due to the automation of the process. The list included the Executive team, but not Directors, so can justly be viewed as 'internal', a key consideration called for by the objectives of the research.

A quick overview of the email list revealed some duplication, e.g. an individual owning both a personal and a positional address. For calculation purposes therefore, a sample size of 350 managers/leaders is assumed. It is recognised that the composition of the sample imposed a clear limitation in that views and evaluation of leadership effectiveness were being sought from the leadership group itself, which may have resulted in more bias than if the questionnaire had been distributed to the remainder of the $\pm 4,700$ employee, 'follower' workforce. The generalisability of the questionnaire findings to the whole workforce and beyond is therefore questionable, although mitigated in part by the triangulation achieved by the use of mixed research methods.

The internet hyperlink to the questionnaire, hosted by SurveyMonkey®, was provided in an email sent to the 'Managers' (Appendix 15). The subject title of the email was 'Leadership in the Health Sector', in the hope that it would spark the interest of the sample of 'leaders/managers' being targeted, as being relevant to them (Williams, 2003). The email text provided a personal introduction, an indication of the purpose of the research, its authority, expected completion time, assurances of anonymity, closing date and contact details.

The closing date for responses was set for Friday 13th June 2014, allowing three weeks for completion. To enable the outcome to be considered valid, the hope was for 30% participation from the 350 invitations, which would provide 105 responses. This response was achieved within the first week after which it slowed significantly. After consulting with researcher colleagues as to the acceptability of such a move, I decided to bring the closing date forward by one week. An email was therefore sent to all invitees after 10 days, thanking those who had already responded, advising that the end date was being brought forward by a week due to the good response. One further reminder was sent out one day before closing. These measures triggered further activity, resulting in a pleasing total of 164 respondents (46.9%)(Appendix 16).

3.8 Analytical considerations

In determining the data to be gathered, consideration was given to how that data would be analysed and therefore, what data would be necessary to facilitate that analysis. Various factors were considered, including confidence levels, sample size, reliability of data, validity of analysis and interpretation, generalisability of findings and so on. The diversity of backgrounds, training

and experience of participants and respondents in the research was also recognised and questions were included to permit cross tabulation of data to isolate views and responses.

3.9 Bias

As internal auditor, the subject material is closely aligned with a key area of my work, so there was always the risk that I might approach the research with preconceived ideas and with a separate agenda. In order to mitigate this, I had to continuously make a conscious effort to 'take off my internal auditor hat, and put on the student, researcher hat'. In fact I recall asking interviewees not to view me as the auditor, but to try and see me as a researcher who was interested in them and what they had to say.

The potential for researcher and participant bias was recognised in respect of each data collection method used, and has been discussed earlier. (Saunders, 2012; Ghauri, 2010; Robson, 2002).

3.10 Conclusion

Given the various constraints on time, budget and dissertation word-count, the methods used provided opportunities for personal, detailed perspectives to be obtained, as well as a broad view from a wide audience that responded well to requests for input. In hindsight however, a measured use of observational research methods may have provided useful evidence for evaluation purposes.

I believe the multi-method approach was appropriate and successful, and the rich findings, both qualitative and quantitative, are presented in the next chapter.

4.1 Introduction

The purpose of this chapter is to present and discuss the findings of the research undertaken to address the two main objectives, which were to obtain an understanding from within the case study organisation, AnHHS, as to what effective leadership might look like and to assess whether effective leadership is evident within AnHHS.

4.2 Data gathered

4.2.1 Interviews

Timing and duration

Ten interviews were conducted over a four-week period and the relatively short time-lapse of 24 days between the first and last interviews ensured a consistent approach in all interviews Table 10.

Table 10: Interview dates and duration

Interview ID Code	Date	Duration (minutes)		
		Pilot	Directors	Executives
T0	8 April	40		
T1	11 April			36
T2	15 April			39
T3	16 April			40
T4	17 April		43	
T5	22 April		47	
T6	24 April		35	
T7	24 April		44	
T8	30 April			44
T9	30 April		49	
T10	2 May			59
	Average	40	43.6	43.6

Audio recordings were made and written transcripts were prepared from the recordings, with lines numbered for ease of analysis and reference.

Table 11: Themes and key words from Cycle 1 coding of interviews
(full schedule of codes – Appendix 10)

Themes	Key words and phrases	Frequency	Respondents
Attributes	Ambition and drive	11	5
	Decisive	13	5
	Honest (don't hide mistakes)	5	4
	Intelligent	8	3
	Passion	5	4
	Patient	2	1
	Self-awareness and understanding	17	5
	Self-discipline	7	3
Skills and knowledge	Culturally aware	4	3
	Experience	13	7
	Knowledge	45	10
	Learns	17	10
	Politically aware	11	6
	Training	22	9
	Understanding	39	8
	Expertise	27	6
	Skilled	27	8
	Strategic thinker	28	8
Behaviour	Coach	9	4
	Communicate	24	5
	Diligent	13	8
	Directs & guides	14	7
	Encourage	13	6
	Engaged	17	7
	Example	7	5
	Inspire	10	5
	Interested	7	4
	Invest in training	5	2
	Listens	14	5
	Mentors	12	6
	Motivation (self and others)	18	8
	Relationship	30	7
	Respects	26	9
	Strong	13	5
	Teamwork	22	7
	Trust	9	4
	Visible	10	5

Coding

Using the transcripts, a first cycle of coding identified themes and key words (Appendix 10), which were drawn upon in the development of the questionnaire for administration amongst a wider sample of the employee population. Table 11 shows the key words that appeared during the interviews, their frequency of occurrence and the number of interviewees who used them.

The second coding cycle identified a number of key thoughts and concepts (Appendix 11), summarised in Table 12. A set of themed 'quotable quotes' from the interviews was also developed (Appendix 12) for use in later analysis and discussion. Interviewee identity is protected by the use of codes e.g. T1 is the Transcript of interviewee 1.

Table 12: Key thoughts and concepts from Cycle 2 coding of interviews
(full schedule of codes at Appendix 11)

Type	Key thoughts and concepts	Frequency	Respondents
Behaviour	Relationships & respect	146	10
Skill	Clinical or not?	94	10
Attribute	Know thyself	84	9
Behaviour	Team learning and teamwork	78	10
Skill	It's a journey	71	10
Attribute	Don't dither, DECIDE!!	67	9
Other	Spot the difference	40	9
Behaviour	Be bold !	33	8
Other	Which lens?	18	8
Other	Contested terrain	11	2
Other	The Board	45	10

As the interviews were all conducted using the same schedule, all interviewees had the same opportunity to discuss and elaborate upon the same questions. Although it became clear that some felt more strongly than others about certain issues, mere numerics did not determine the key themes, but rather the matters that seemed most likely to have an impact on the effectiveness of leadership. Most interviewees had views about all issues, although the concept of contested terrain was only raised by 2 participants, and is discussed later.

4.2.2 Questionnaire

The questionnaire (Appendix 13) was developed using concepts, principles and key words identified from the literature and the interviews, as described above.

The survey was administered by using SurveyMonkey®, which has inbuilt analysis tools such as charts and graphs, with the capacity to compare and cross-tabulate data. Most questionnaire findings are presented in charts, which, I believe provide the quickest and clearest grasp of the findings, although the numeric data is tabulated in Appendix 17 for any clarification required.

Towards an internal definition of leadership

One of the objectives of this research was to obtain an internal definition of leadership within AnHHS, and questions in Section B sought respondent views on the relative importance of various leader attributes, skills and knowledge, and behaviours.

The evaluation

Another research objective was to determine whether the leadership on display in AnHHS could be considered effective in terms of general definitions, as well as the internal definition derived from this research. Questions in Section C provided respondents with the opportunity to evaluate AnHHS leadership against the definitions that they had earlier determined.

Section D provided respondents with the opportunity to evaluate themselves within the same framework of questions. This is discussed in section 4.3.6 below.

Sample and response

From the sample of 350 invitees, 164 responses were received, resulting in a response rate of 46.9% (Appendix 16). This was considered acceptable in light of trends in organisational research that indicated the average response rate for studies that collected data from individuals was 52.7% with a standard deviation of 20.4 (Baruch & Holtom, 2008). All responses were usable, with relatively few instances of unanswered, or 'skipped' questions.

General data

Section A in the questionnaire (see Appendix 13) sought personal information and demographics from the respondents (Figures 4, 5 and 6) for purposes of gaining an impression of the individual as well as providing the basis for cross-tabulation with data to be obtained in later sections.

The general profile and diversity of respondents indicated a broad range of ages, seniority and backgrounds both within and outside the health sector. Over two-thirds of managers are female, and only about half of respondents with clinical training are still active in clinical roles.

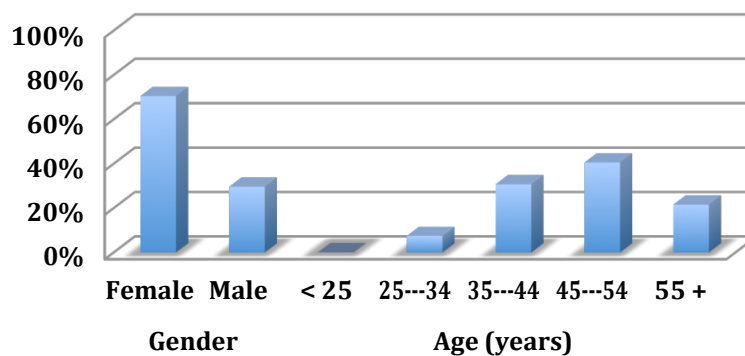


Figure 4: Respondent data - Gender and age

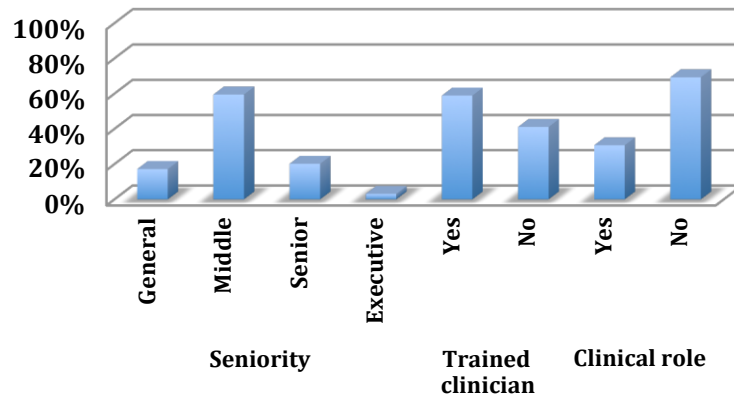


Figure 5: Respondent data – Seniority, training and role

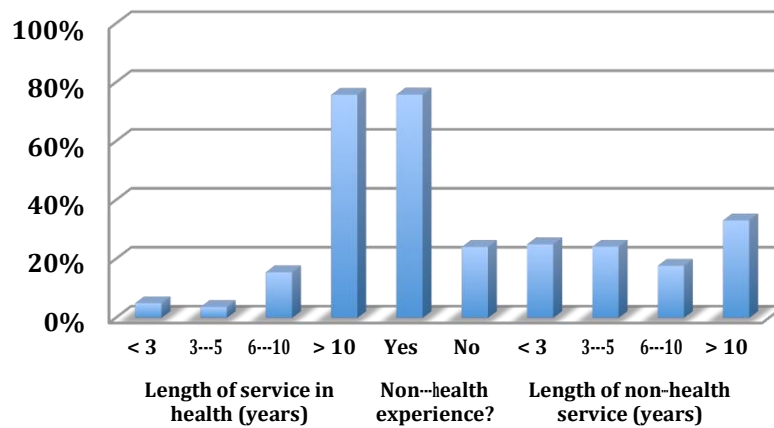


Figure 6: Respondent data – Service and experience

4.3 Analysis and discussion

Using the themes and concepts that arose from the interviews, this section is grouped into Attributes, Skills, Behaviour and Other. The research sought to identify the factors that would both define effective leadership within AnHHS, and the extent to which it is evident. The following discussion combines the findings from the literature, interviews and questionnaires.

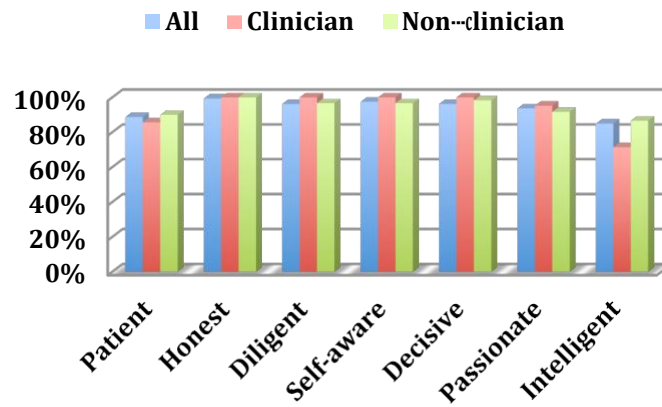


Figure 7: Percentage agreement that the ATTRIBUTES are important

4.3.1 Attributes

As expected, there is a common view throughout, of the importance of these attributes in leaders, regardless of the sphere of operation, although it was a little surprising that fewer clinicians view intelligence as important, than other groups (Figure 7). Intelligence was not defined in the questionnaire and it is assumed that respondents interpreted it to mean *intellectual* intelligence. Perhaps a distinction between the various types of intelligence, e.g. intellectual, emotional, social etc. (Goleman, 1998), would have yielded different results.

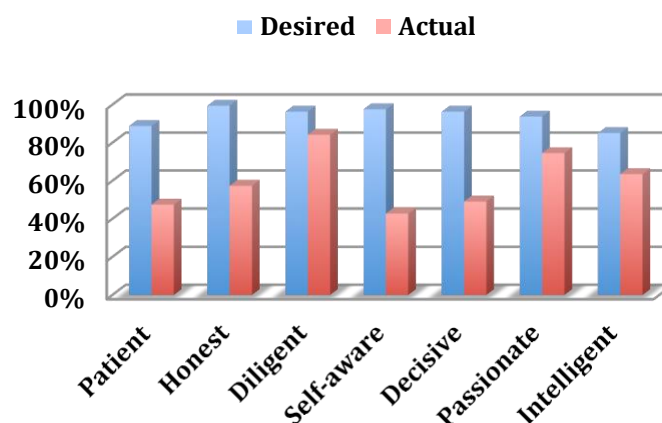


Figure 8: Comparison of percentage who agree that the ATTRIBUTES are important with percentage of those who say they EXIST within AnHHS

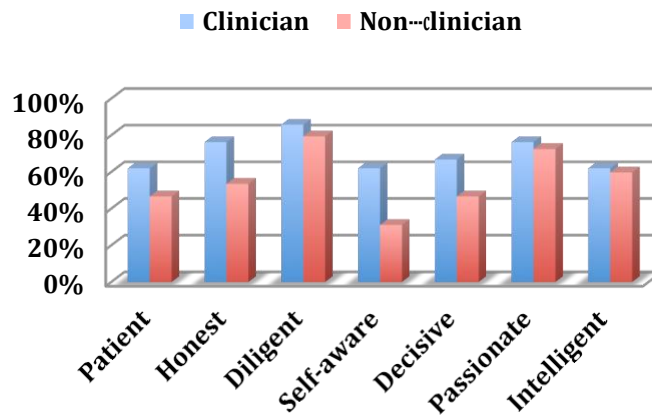


Figure 9: Comparison of CLINICIAN and NON-CLINICIAN evaluation of the EXISTENCE of ATTRIBUTES in leaders of AnHHS

More clinicians see the existence of the personal attributes than do non-clinicians, particularly as regards patience, honesty, self-awareness and decisiveness (Figures 8 and 9). This may have something to do with the clinical nature of their work and what is required of them. Figure 9 suggests that leaders in AnHHS do not meet expectations in terms of attributes.

Know thyself!

The recognition of one's own failings and weaknesses emerged from the survey as being a key feature of good leadership, confirmed by T5 who suggested, "...a good leader knows and is honest with himself". This aligns with the literature on the management of self-regard (Bennis, 1997), an accurate self-assessment (Goleman, 2000), the achievement of personal mastery (Senge, 2006), and the personal qualities expected within the health sector (NHS, 2006).

Various other concepts like humility and servant-hood were also mentioned in the interviews.

Don't dither, DECIDE!

96% of respondents agreed that decisiveness was important, but its existence in AnHHS was only seen by 49% of respondents. More clinicians saw decisiveness than non-clinicians, which indicates a difference in the perspectives from which the issue was viewed, the context in which they perform their duties, and the leaders that they observed and probably based their evaluations upon.

The decisiveness of doctors in clinical matters, described by T2 who said “...you make the best decision with the information you have, and you get on with it...otherwise, the patient will expire in front of you”, was contrasted with the vacillation, or seeming indecisiveness that clinicians sometimes see in business decision-making.

The distinction between the quick, ‘think on your feet’ decision-making expected of clinicians, and the informed, well thought-through decisions (seen as vacillation by clinicians) made by organisational leaders might explain the diversion of views, and was identified by CCL (2010) research within the health sector, which found that ‘quick and approximate’ was preferred over ‘slow and precise’.

It was noted, however, that the evaluation by clinicians currently engaged in non-clinical roles was closer to that of non-clinicians, where decisiveness was not seen to be as crisp. Perhaps the ‘life and death’ urgency typical of some clinical decisions was not deemed essential for the considered and strategic decisions of corporate roles.

4.3.2 Skills and knowledge

As with the finding regarding attributes, there is general agreement in all groupings as to the importance of all skills and knowledge factors, although the relative importance of the skills is quite varied. Significant importance is placed on sector knowledge, skill and expertise, lifelong learning and continuous improvement, cultural awareness and strategic thinking, with least importance being placed on short-term thinking (Figure 10).

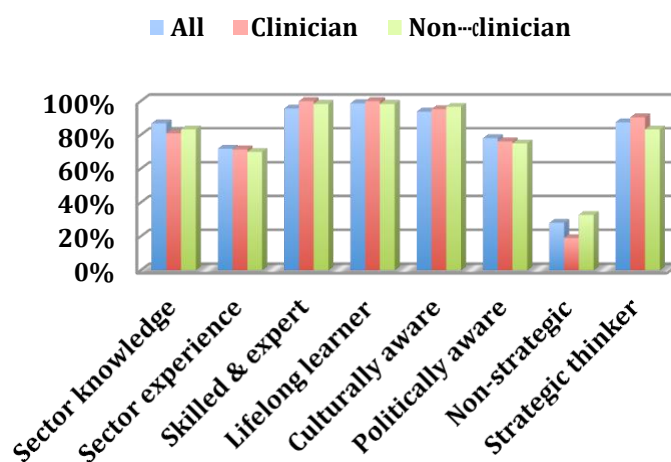


Figure 10: Percentage agreement that SKILLS and KNOWLEDGE are important

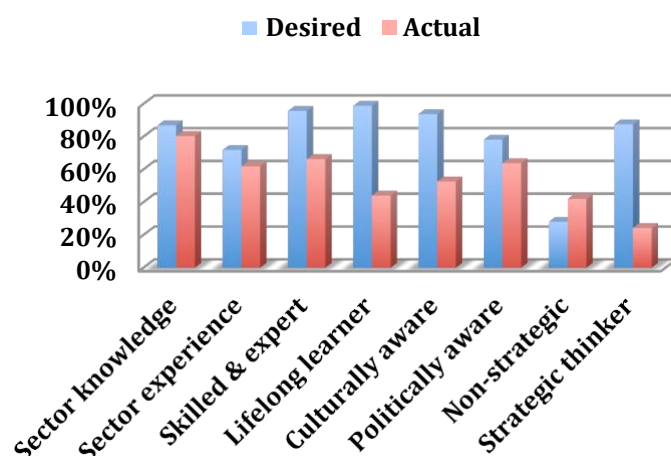


Figure 11: Comparison of percentage who agree that the SKILLS and KNOWLEDGE are important with percentage of those who say they EXIST within AnHHS

There are large gaps between the desired and evident skills (Figure 11), particularly the most significant ones related to expertise, continuous improvement, cultural awareness and strategic thinking.

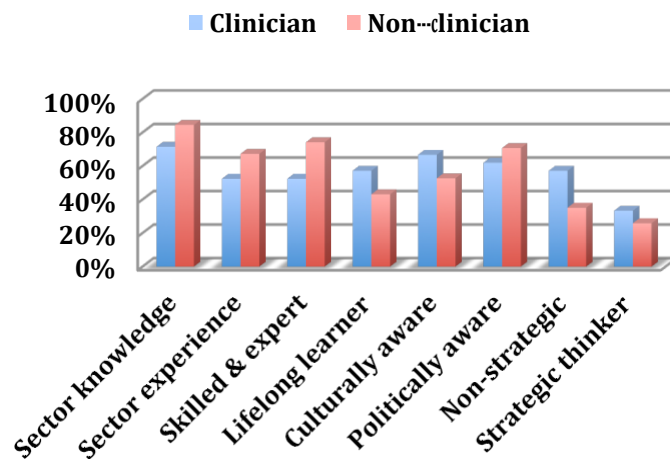


Figure 12: Comparison of CLINICIAN and NON-CLINICIAN evaluation of the EXISTENCE of SKILLS and KNOWLEDGE in AnHHS leaders

Clinical or not?

The clinical background of a leader in the health sector is often a consideration when credibility is at stake and the effectiveness of leadership is brought into question. No consistent position was found, with comments ranging from ‘...you need a clinical background to manage a clinical service’ (T3), to a less harsh ‘...it is much harder for people without a clinical background to establish credibility...’ (T1) to the very neutral view expressed by T5 who said ‘I think you can lead in health care having not been in health at all.’

The questionnaire was not incisive enough to be able to obtain a clear distinction between the importance and existence of the skills of clinicians and non-clinicians. Although it is specific to the healthcare sector, the NHS (2006) Leadership Qualities Framework does not identify this distinction either.

It's a journey

With a weighted response of 98.8% of all respondents, lifelong learning was rated as the most important skill desired in the health sector, with 100% of clinicians agreeing and 98.3% of others.

With this skill being described variously as conscientious (Goleman, 2000), drive for improvement (NHS, 2006), personal mastery (Senge, 2006) and continual learning (Burke, 2011), it is clearly rated very highly as being key for effective leadership. The learning need not be only academic, but of any aspect of leadership and life. Wise man, King Solomon wrote, "To learn, you must want to be taught. To refuse reproof is stupid." (Proverbs 12:1. The Living Bible).

Unfortunately, only 57% of clinicians and 43% of non-clinicians indicated that there was evidence of this leadership skill within AnHHS (Figure 12). I don't believe this is calling for a mass enrolment in courses, but is an evaluation of the willingness to be open to new ideas (see Q11d in Appendix 13). Without imputing this as a finding, it may simply be an expression of frustration with bureaucracy and the 'we always do it like this' syndrome, rather than dealing with the issue raised by T10 that "...a big challenge in leadership is that it is easier to lead when things are going well than when it is all falling apart."

4.3.3 Behaviour

Again, there is consistency in the views of all groups as to the importance of leadership behaviours. It seems, however, that those with a non-clinical background are less 'cuddly' in terms of taking a personal interest, and possibly forming personal relationships with staff and

their families. All are agreed on the inappropriateness of the negative behaviour of a tough, forceful style of leadership within this context. (Figure 13).

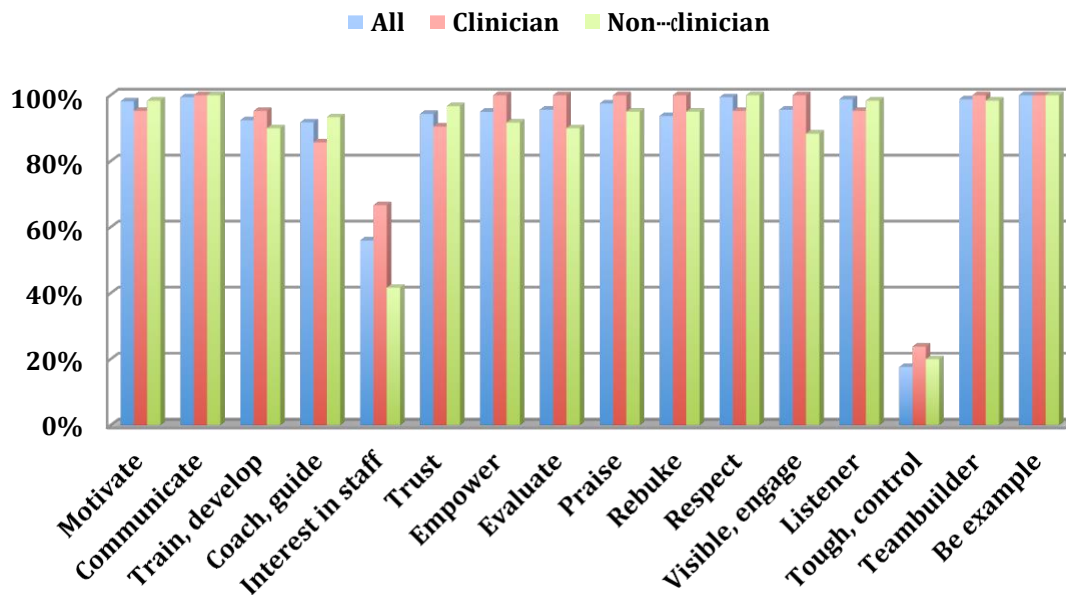


Figure 13: Percentage agreement that the BEHAVIOURS are important

The evaluation of behaviours within AnHHS shows the most significant gaps between the desired state and that which actually exists (Figure 14). In fact the averages show that 89% of respondents desired the behaviours whereas only 37% rated them as being evident (Appendix 17).

Whilst more clinicians see evidence of every single aspect of behaviour than non-clinicians, the consistently poor ratings are clear (Figure 15). It is only in the area of toughness and forcefulness that both clinicians and non-clinicians see similar levels of evidence of this undesired behaviour. Unfortunately, it is in the areas of staff development, such as motivation, communication, training, coaching, evaluation of work and dealing with poor performance, (Avolio et al. 2003), that non-clinicians rate health leaders more poorly than their clinician colleagues (Figure 15). Again, this may be related to both the context in which these behaviours are actually seen by respondents, and the category of leaders being evaluated. Perhaps too, in a

hospital environment, non-clinical staff development is not seen to be as important. Again, this research was not incisive enough to be conclusive.

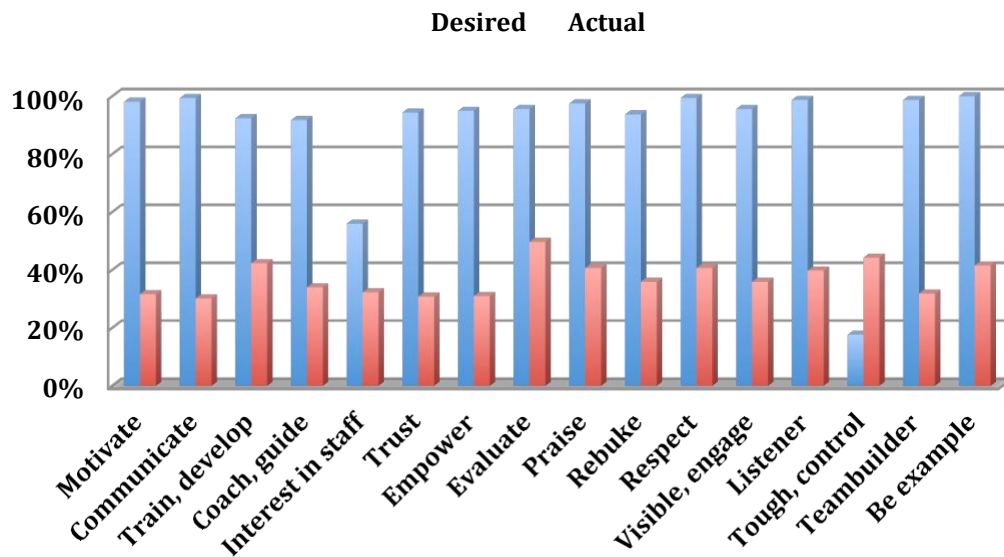


Figure 14: Comparison of percentage who agree that the BEHAVIOURS are important with percentage of those who say they EXIST within AnHHS

Relationships and respect

This theme, which encompassed elements such as respect, trust, empathy, ethics, communication and so on, was mentioned 146 times by all participants and was a key interview discussion point, with one respondent suggesting that “...leadership is fundamentally connected to relationships...” Goleman’s (2000) model points to such ‘social skills’ as key components of Emotional Intelligence.

One doctor, with business experience, was appalled that “...in order to succeed in the private world it seems you have to lie and cheat your way through...” He acknowledged that he might have been naive to think that the ethics and integrity so ingrained in clinicians was to be found outside the health sector. Of course, this may well be one person’s unfortunate experience.

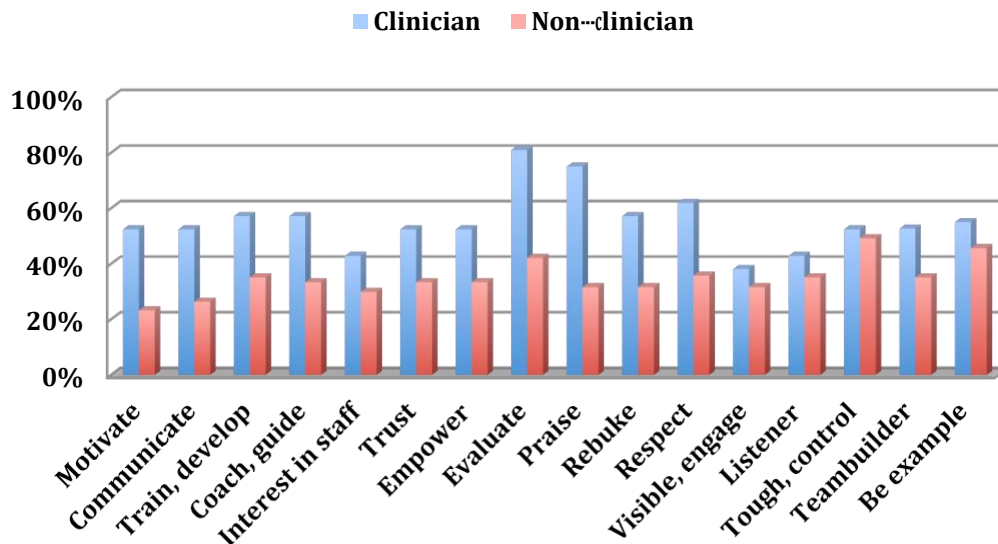


Figure 15: Comparison of CLINICIAN and NON-CLINICIAN evaluation of the EXISTENCE of the BEHAVIOURS in AnHHS leaders

Team learning and teamwork

When asked about the effectiveness of leadership in AnHHS, T1 measured it against achievements: “...through a difficult period of change and resource constraints, as a team we’ve kept people safe, provided good service and are building a strong culture.”

A unity of thought and determination was evident from all the executives. There was a strong sense of *team-ness* where “...we’re learning together, people from different clinical backgrounds...but I think it’s very useful.” (T1).

Be bold!

A feature of good leadership is drive and ambition (Collins, 2001; NHS, 2006), with a passion for work, where you would say “...give me a chance of running it...” (T2) and then by “...taking on bigger and bigger places...your leadership goes to different levels” (T5). It seems however, that

boldness is sometimes enacted through a 'tough and controlling' approach, which is not desirable (Figure 14).

4.3.4 Other

Contested terrain

Although only raised by 2 participants, this interesting concept emerged and refers to ownership of the client. One respondent assessed that "...in other sectors... everyone spends their time avoiding owning the client...[whereas]...here, people fight over who owns the patient. Nurses compete with who can do what, and doctors say they can do this and it's hierarchical, a contested domain." (T1).

A further distinction was drawn that in health there are many practitioners and experts all focused on the needs of a single patient, whereas in other sectors, like education for example, there are many students whose needs are being met by one teacher i.e. the difference between the 'many-to-one' versus the 'one-to-many'.

Spot the difference

In trying to identify whether there were any significant differences between the health sector and the non-health sector, it became evident that the health sector consists of several components in itself, significantly, the private and the public sectors, as pointed out by T8 who said "...Australia operates a two-track health care system where 60% of all surgery is now done in the private sector."

Nevertheless, a major difference, particularly as it affects leadership, is the intelligence, education and diversity of professions, with T10 saying “the thing about health is the high intelligence of the workforce...[and the difficulty in leading] a bunch of independent, free-thinking, highly educated, highly trained, well-paid staff.”

T5 suggested that “...probably no other industry in the world has so many different groups of professionals in it.” This resonates with the contested terrain concept above, where many professions are engaged with one patient.

Which lens?

When asked for an assessment of the effectiveness of leadership it became clear that it is not a single-dimensional answer and that “effectiveness can be viewed through a number of lenses...community, resources, finance, quality and safety, reputation, so you need a framework.” (T1).

This issue was identified in the discussions on decisiveness and skills above.

4.3.5 Interviewee evaluation of leadership effectiveness

Interviewees were asked to give their view on the effectiveness of leadership within AnHHS, and generally the response was that it was effective when measured by achievements as follows:

T1: “I think our leadership is effective...through a difficult period of change, resource constraints and as a team we’ve kept people safe, provided good service and are building a strong culture.”

T7: "...yes, I think it is effective...but there is always room for improvement, in the sense of listening...and being seen."

T8: "...yes, we've now made a profit...we've eliminated waiting lists...we're moving towards achieving national emergency access targets."

T8: "I believe it's chalk and cheese...and so I think that from a management point of view, we must have been effective."

T10: "Clearly the leadership team has been effective. Look at the results: surplus...savings... balanced the budget...zero long-waitlists...met national targets...doing more activity ... maintaining safety and quality..."

It is interesting to note that during the interviews, effectiveness was discussed in terms of attributes, skills and behaviours and what effective leadership might look like, but when asked if leadership was effective, every interviewee reverted to the factors that Kotter (1990) describes as management, not leadership. The leadership factors mentioned in Table 1 in Chapter 2, such as aligning people, motivation and inspiring people, establishing direction and so on, are not mentioned at all, although perhaps their existence might explain the described achievements.

Another factor may be the difficult circumstances facing AnHHS at the outset, which demanded the coercive and authoritative styles identified earlier, that have been perceived in this evaluation (Goleman, 1998). Perhaps, in a longitudinal research, a move towards the more sustainable styles like affiliative, democratic and eventually coaching may be perceived.

Charts have been presented in the discussion of various aspects of leadership and Table 13 presents a summary of the 'Top 5' attributes, skills and behaviours in terms of importance to all respondents to the questionnaire.

These matters have been discussed individually above, but this table indicates that respondents do not believe that existing leadership within AnHHS measures up to what is considered important.

Table 13: Percentage of respondents that:
1) rated the quality as important (DESIRED); and
2) considered it to be evident in AnHHS (ACTUAL)

Quality		Desired	Actual
Attributes	Honesty	99.4%	57.3%
	Self-awareness	97.5%	42.7%
	Diligence	96.3%	84.1%
	Decisiveness	96.2%	49%
	Passion and drive	93.7%	74.5%
	Average	96.6%	61.5%
Skills and knowledge	Lifelong learner open to new ideas	98.8%	43.9%
	Skill and expertise	95.6%	66.2%
	Cultural awareness and sensitivity	93.8%	52.6%
	Strategic thinking	87.5%	24.2%
	Sector knowledge	86.9%	80.3%
	Average	92.5%	53.4%
Behaviours	Lead by example	100%	41.6%
	Respect	99.4%	40.6%
	Communication	99.4%	30.1%
	Team builder	98.7%	31.8%
	Listener	98.7%	39.7%
	Average	99.2%	36.8%

4.3.6 Self-evaluation by questionnaire respondents

This matter is discussed separately, so as not to confuse the findings of the research in terms of definition and existence of leadership within AnHHS.

The purpose of Section D of the questionnaire was to determine the extent to which self-evaluation aligned with general evaluation of overall leadership.

In all attributes, respondents rated themselves higher than the overall rating (Figure 16) and in many instances at close to the internal definition of effective leadership itself.

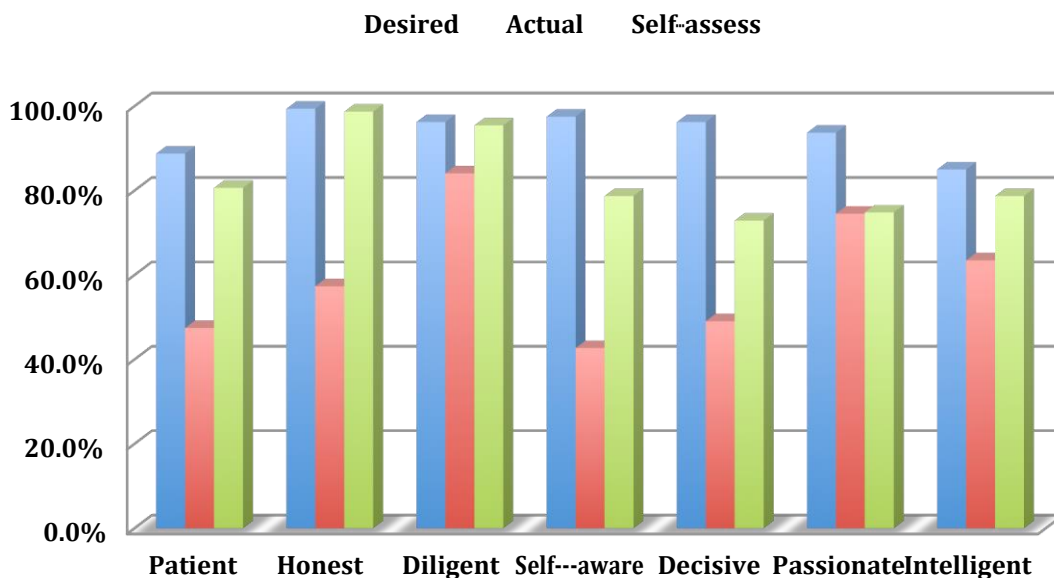


Figure 16: Self-evaluation - ATTRIBUTES

Respondents rated themselves highly in terms of expertise, lifelong learning and cultural awareness, and far greater than the overall rating in these as well as strategic thinking (Figure 17).

In terms of behaviour, respondents again rate themselves at close to ideal in nearly every respect, and in all respects, substantially better than the overall rating of leadership within AnHHS (Figure 18).

It may have been an unrealistic expectation, but I had somehow believed that the respondents, who represented the very leadership group under evaluation, would rate themselves similarly to the way the organisation's leadership had been rated. This would have given me comfort that the self-awareness most managers claimed for themselves was in fact a reality. As it happens, this was not the case.

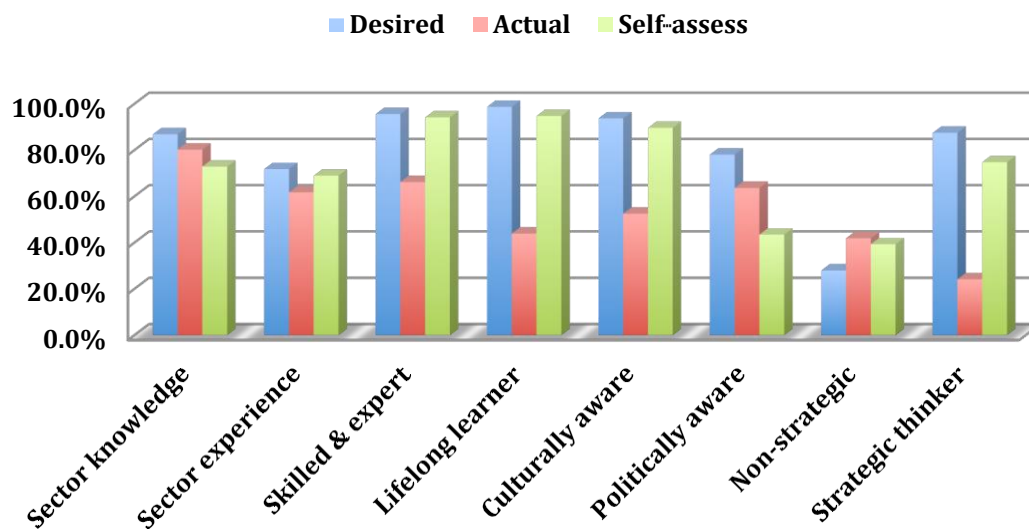


Figure 17: Self-evaluation - *SKILLS and KNOWLEDGE*

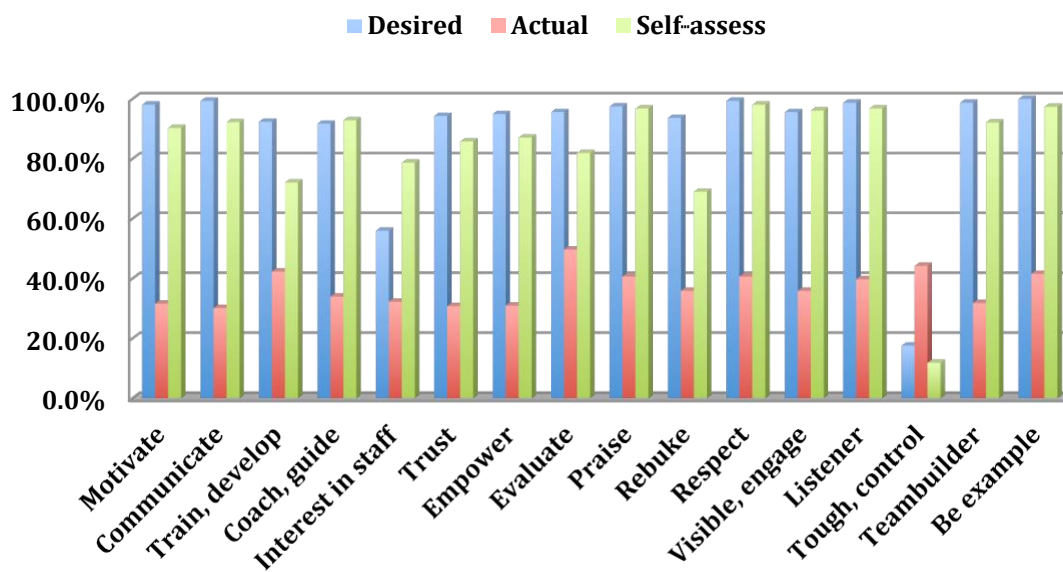


Figure 18: Self-evaluation - *BEHAVIOURS*

4.4 Conclusion

This analysis of findings has been enlightening in that the general tone of the interviews was reflected through the questionnaires in terms of identifying what effective leadership might look like within AnHHS. However, when it came to the evaluation of the current effectiveness of

leadership within AnHHS, the findings from the questionnaires and the interviewees differed. The explanation may lie in contingency theory, which is a subject for further research.

The next chapter will discuss the conclusions that might be reached from this research.

5.1 Introduction

This dissertation posed the question, ‘What makes leadership effective within a Health Service?’ Answers were sought by searching the literature, deriving an internal understanding of leadership within AnHHS, and then assessing the existence of leadership as it is understood.

This chapter presents conclusions drawn, which are based on the research carried out, and then offers some recommendations for the consideration of AnHHS leaders.

Recognised limitations of the research and the extent to which it can be generalised are also pointed out, and lastly, opportunities for further research that were identified during this project are suggested.

5.2 Conclusions

5.2.1 Internal meaning and understanding

Conclusion 1

Arising from the review of literature and the interviews, certain themes and topics were incorporated into the questionnaire for engagement with the wider sample. These mirror the emphases placed by various models, such as the Leadership Qualities Framework (NHS, 2006) and the Kouzes and Posner’s (2012) Exemplary Practices model.

There are different types of leadership found in healthcare organisations, and this research has identified at least two: clinical and non-clinical. However, the research gathered sufficient evidence to present the top 5 attributes, skills and behaviours as an internally derived definition of effective leadership within AnHHS, according to respondents in the research.

Effective leaders within the healthcare sector:

- Are: honest; self-aware; diligent; decisive; passionate and driven;
- Have: a lifelong learning mindset and attitude; expertise in their field; sensitivity towards, and an awareness of cultural, political, religious and other differences; the ability to think strategically; a good knowledge of the healthcare sector; and
- Behave: in a way that sets the desired tone and gives a good example; respectfully towards others; with clarity in frequent communication; as a team player and a team-builder; by actively listening in a way that people feel heard (Appendix 18).

Given the range of themes and concepts to choose from, the accord with general leadership literature, and the extent of agreement from all respondents as to their importance, I conclude that there is a general understanding within AnHHS of what effective leadership means.

5.2.2 Existence

Conclusion 2

Although a level of understanding of the meaning of effective leadership can be seen in AnHHS, this understanding does not translate to evidence of its **current** existence.

However, the type of leadership that does seem to exist resembles the coercive and authoritative end of Goleman's (2000) continuum of leadership styles, which in the right circumstances might be considered as effective. Of course, as much as the staff may not have liked it, that leadership approach may have been the most appropriate for AnHHS for the season immediately following the 'birth' of the independent organisation. AnHHS is on a journey through appropriate styles of leadership, but leadership groups may be slow to transition to the next stage along Goleman's (2000) continuum.

I conclude therefore, that, seen through the lens of goal achievement, effective leadership *has existed* within AnHHS, but the current style of leadership is losing its momentum and effectiveness. However, when viewed through the *defined and desired* lens, effective leadership does not currently exist in AnHHS. Leaders need to discern the appropriateness of a particular leadership style, and to remain effective by adapting accordingly.

Conclusion 3

AnHHS operates within the social sector, meaning that its drivers are not the same as the general business sector Collins (2006)(Appendix 4). Within AnHHS itself, are functional streams that experience a similar tension e.g. the finance function striving for economies and efficiencies, whereas the professional, clinical functions just wanting to deliver the service without being burdened with corporate 'bump'. This tension is evidenced by the significantly different ratings given by clinicians and non-clinicians as to the existence of effective leadership within AnHHS.

I conclude therefore, that within a diverse organisation such as AnHHS, the situational leadership described by Yukl (2013) is a reality. The movement along the aforementioned

Goleman continuum may be at a different pace from function to function, and this is the case within AnHHS. Executive leadership must be aware of these nuances and respond accordingly.

5.3 Recommendations

Arising from the research, certain recommendations for improving the effectiveness of leadership within AnHHS are presented for consideration by the executive team, together with suggestions on how some may be implemented.

Recommendation 1

Organisational leaders may have been subconsciously aware of the style that was necessary at the outset, and implemented it effectively. However, its time has passed, and an increasing awareness must now occur, of the change in leadership style that needs to take place.

Implementation:

- An internal model should be developed as part of a 'leader training' module, that increases cognisance of different leadership styles, along the lines of Goleman's (2000) continuum discussed in Chapter 2;
- Self-evaluations should be carried out by individual leaders to determine 'where they are' in this model;
- Development and training should be designed for leaders, and incorporated into their performance and development programmes.

Recommendation 2

An internal staff survey should be conducted to gain a more accurate understanding of the perceptions of leadership behaviours, with a view to improving upon shortcomings.

Implementation:

- Identify the lenses through which the desired understanding can be obtained, e.g. clinical, executive, organisational unit/facilities, executive, Board etc.;
- Develop *lens-appropriate* questionnaires for administration;
- Communicate the intention to conduct this survey clearly, with the improvement objective, and the organisational commitment, clearly articulated;
- Conduct the survey over a convenient period;
- Intentionally work on improving the behaviours of leaders by incorporating it into the performance appraisal and development processes.

Recommendation 3

Conduct a 'strengths' audit to determine the strengths of all leaders and staff, with a view to aligning roles with strengths where practicable and possible, rather than investing resources in addressing weaknesses (Buckingham, 2005).

Implementation:

- Implement an organisation-wide project, facilitated by organisational units and teams, to identify staff and leader strengths by using the Clifton Strengthsfinder programme that can be found at www.strengthfinder.com;
- Build into the performance and development appraisal process and organisational workforce planning strategies, an element to identify and develop staff strengths, and to align them with roles where possible.

Recommendation 4

Organisational units should all be encouraged to conduct an evaluation exercise whereby all team members carry out a self-evaluation, as well as evaluating all other team members. This will result in clearer personal understanding, as well as that of other team members.

Implementation:

- Customise the Blake and Mouton Leadership Grid (Northouse, 2013), which is very simple to use and administer, for AnHHS purposes;
- Over a designated period of time, say one month, require all staff to conduct self-evaluations and evaluations of their immediate team colleagues, by using Bass' (2008) Multifactor Leadership Questionnaire;;
- Allow time for evaluation and discussion at team level;
- Prepare enhancement and development plans.

Recommendation 5

Leveraging off the outcome of Recommendation 4, teams should seek to improve performance at various levels such as executive, functional, organisational unit and department, in order to improve team perspective and to encourage leadership thinking outside one's own functional area.

Implementation:

- Leadership teams should set aside regular times for teambuilding;
- honest and open evaluation and discussion should be encouraged;
- objectives and steps towards improvement should be identified;
- achievements should be monitored, managed and celebrated.

5.4 Limitations

Time, financial budget and assignment word-count constraints have meant that only a very general picture of the meaning and existence of effective leadership within AnHHS could be obtained.

Interviews with a wider range of staff and an organisation-wide survey may have revealed different perspectives, and potentially a different definition and assessment of leadership within AnHHS.

The use of only two primary research instruments, interviews and a survey, has potentially limited the validity of this research. For example, a broad, longitudinal observational research of 'leaders in action' within different contexts and over a period of time to ensure consistency as opposed to 'playacting for the auditor', may have allowed a more objective assessment of the definition of leadership and the evidence of its existence.

5.5 Generalisability

The findings of this research arise from a relatively limited range of views and perspectives (<400 people out of ±4700), and may not reflect the views of all staff throughout the whole organisation. They should therefore not be generalised within AnHHS itself.

Having said that, I would not be surprised if similar research in other Queensland Hospital and Health Services arrived at the same conclusions. The research may therefore be generalisable to a limited extent in these bodies, but certainly not the healthcare sector at large.

5.6 Opportunities for further research

There is definitely more to be learned about leadership within the healthcare sector, and this learning could be assisted by the following research opportunities, framed as research questions, that became evident during this project:

- What are the differences between leadership in the private healthcare sector and the public healthcare sector? (see section 4.3.4);
- Is situational leadership relevant in the healthcare sector? This could consider the clinical/non-clinical operational contexts as well as organisational contexts that call for different leadership styles (see section 4.4);
- What does effective Board leadership and governance within the healthcare sector look like? (see sections 3.7.2 and 4.3.5).

These matters have either been recognised or touched upon during this research, and although some of these projects could be done over a short period of time by cross-sectional study, others lend themselves more to longitudinal research for added meaning (see section 3.4.2).

5.7 Conclusion

Leadership within AnHHS is evolving, as is the maturity of the organisation. This work indicates where AnHHS is, where it should be, and offers a few pointers on how to get there.

6.1 Introduction

This chapter presents my reflections on this module and the achievement of its objectives, the research process undertaken, and my learning as well as identifying areas for my further personal development.

6.2 Module objectives

The stated aim (Module Summary) of this dissertation module MOD053 is to provide an opportunity to undertake independent research, and sets out a number of objectives to ensure its accomplishment. Table 14 provides a summary of these aims and the evidence that they have been attained.

Table 14: Description of module objectives and achievements

Objectives and Learning Outcomes		Fulfilled? (Yes/No)	Evidence of work undertaken to meet course objective
No.	Description		
1.	Define an area for research and establish a research question related to innovation, change and leadership	Yes	Selected the area of 'Leadership within the Healthcare Sector', aligning it with a core component of MALIC – Chapter 1.
2.	Critically evaluate a range of appropriate methodologies for data collection	Yes	In Chapter 3, a range of methods was considered, some used, some started then ceased, and some rejected.
3.	Submit a research proposal to the University research ethics committee.	Yes	Submitted at end of Month 1.
4.	Critically evaluate approaches to collection and analysis of qualitative and/or quantitative data	Yes	Discussed in Chapter 3 – Research Methodology.
5.	Analyse and present research data in clear ways, to inform and interest the reader	Yes	Chapters 4 presents Findings, Analysis and Discussion; Chapter 5 presents Conclusions derived.
6.	Construct a conceptual framework which integrates theory and practice	Yes	Developed in Chapter 3 – Research Methodology.

6.3 Personal skills acquired

Prior to this program, and in some respects, this module, I did not have the skill to perform a number of the tasks required for this dissertation.

Critical review of literature

I was usually content to read literature on various subjects, and assume that the author, being a published researcher and writer, is an authority on the subject and accept what is written at face value. This course has taught me to think more laterally, to measure what is said against other evidence or opinion and form my own views. Chapter 2 indicates to me that I have made a measure of progress and developed this skill to an extent, but have significant room for improvement.

Research

Prior to this programme I had never before engaged in research. Some aspects of the research were totally foreign, such as coding and qualitative analysis. Any perceived skill as a result of this dissertation is evidence of my learning.

The thought processes, structured methodologies, analytical thinking, written discussion, drawing of conclusions and eventual presentation are all skills either learned for the first time, or honed through this dissertation process.

Although I had some previous experience in some of the methods also used in research, such as interviewing and numeric analysis, as a result of my financial background, these skills were significantly enhanced through this process and the exercise of these skills is now more intentional.

From supervision to self-supervision

Any study that I have previously undertaken has been structured and under the close supervision of a teacher, lecturer or tutor. At the outset, I struggled with making my decisions and working out how to proceed, but with the guidance of my supervisor I have been able to develop self-supervision, map my own plan of action and manage its progress.

6.4 Development opportunities

From a researching point of view, there are a number of areas in which I require further development, such as:

- the use of technologies to access databases and other sources of literature and knowledge;
- more criticality in my review of literature;
- increase knowledge and practice in the area of qualitative analysis e.g. coding;
- analysis, discussion and derivation of valid conclusions from evidence;
- my academic time management
- organisation of data retention and filing;
- continued improvement in my interviewing skills.

6.5 Conclusion

As a first research experience, this has been interesting, exhausting, fulfilling, frustrating, scary and enjoyable – if all that is possible at the same time!!

Would I do it again?

My wife would say “Please don’t!”

As for me... not in a hurry, but it’s not out of the question.

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APPENDIX 1

Extracts from the Queensland Government health reform strategy

Source: State of Queensland (Queensland Health) [2013]. Blueprint for better healthcare in Queensland

Premier's message

A statewide healthcare system with new capacity, co-operation, transparent reporting systems, financial accountability and with patients the focus of attention – this is a vision all Queenslanders want to see.

From its first day, this government has worked long and hard to make lasting health improvements a reality for Queensland families.

Queensland is a great state with great opportunity. This Blueprint for better healthcare in Queensland is the action-plan that will move the Queensland healthcare system from the first phase of repair to lasting recovery. It will transform a struggling healthcare system that fell too far behind into a model for productivity, care and efficiency to meet and surpass national benchmarks.

The blueprint includes a long list of changes that will be instantly recognised by patients, local communities, doctors, nurses and healthcare workers. This is a system no longer constrained by historical limits to patterns of service delivery.

In all cases, the new system will work to provide Queensland communities improved access to services. Challenges, such as limited access to staff and resources in remote areas, will be addressed under the blueprint with new options and alternative solutions.

In remote communities, this government will work to provide 24-hour access to safe and sustainable care through a revised network of Tele-health facilities for the very first time.

This is a 21st century solution to a problem long-regarded as impossible. Our plan will provide reliable health services in places where they were never previously available.

Hospital in the Home, the practice of providing home-based care at hospital standards, is another priority, as is the creation of an independent Mental Health Commission for Queensland.

Across the state, Hospital and Health Services will switch their attention from measuring inputs to reporting patient outcomes. For the first time, their performance—at improving emergency department and surgery waiting times, for example – will be pro-actively published regularly in local newspapers.

This is the open and accountable health system that will rebuild the confidence of every Queenslanders.

The government is working hard for Queenslanders. In less than 12 months we have seen up to 15 per cent improvement in key performance indicators for our major hospitals.

By providing the best services, at the best time and in the best place, we can do even better.

Campbell Newman

Premier of Queensland

Minister's foreword

A healthcare system for generations to come

Queensland is a great state with a good healthcare system, but there are many opportunities for better performance. The Blueprint for better healthcare in Queensland sets the scene for structural and cultural improvements in a health system we will all be proud of.

In the past, this state was the national pace-setter in healthcare. Long before free hospitals appeared on the national agenda, they were a basic entitlement of Queenslanders.

Twelve months ago the Independent Hospital Pricing Authority ranked Queensland second-last among mainland states when it came to the efficient provision of healthcare services.

This is the big task of repair we began in 2012. Having a new State Government and better management is just part of the answer. Better modes of delivery and a more intelligent use of resources are key strategies to expand services and improve performance. Already the key performance indicators are turning around.

Another determining factor is culture. Good workplace culture and leadership in our hospitals and health services is essential. It frames the recommendations adopted by the government to address ambulance ramping and forms the basis for our programs of clinical redesign.

Good culture places a high value on scarce health resources, values our fellow employees and puts patients first.

Queenslanders expect high-quality healthcare whenever it is needed. An accurate understanding of the performance of the health system should be common knowledge. These expectations require continuous improvements in service delivery, no matter where people live.

The Blueprint for better healthcare in Queensland has four principal themes:

1. Health services focused on patients and people.
2. Empowering the community and our health workforce.
3. Providing Queenslanders with value in health services.
4. Investing, innovating and planning for the future.

Getting value from every health dollar, encouraging collaboration and providing for future needs through investment, innovation and planning will return Queensland's healthcare system to lead the nation.

Lawrence Springborg

Minister for Health

Hospital and Health Services

The government established 17 statutory Hospital and Health Services (HHSs) in July 2012. Hospital and Health Boards are answerable to the Minister for Health and accountable to the local community.

Orientating health services to better meet local health needs is a priority for all HHSs. This requires significant change to many of the established cultures and practices that impact on performance and will take time to complete.

HHSs are no longer constrained by historical patterns of service delivery. Within the context of statewide planning objectives and local factors such as geographic location, workforce supply and access to infrastructure and equipment, they have flexibility to consult the community and determine the best mix of health service.

Under a devolved system of governance, the role of the State Government is to provide boards with the full range of legislative and regulatory tools that they need to accomplish their task.

Source: State of Queensland (Queensland Health) [2014]

Source: State of Queensland (Queensland Health) [2014]

Hospital and Health Services, Queensland and Health
by Recognised Public Hospitals



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APPENDIX 3

Alignment of Level 5 Leadership with CCL research findings

Sources: Developed from CCL (2010) and Collins (2001)

CCL research			Collins (2001) alignment
Rank	Skill	Description	
1	Leading employees	Delegates, creates employee opportunities, fair, hires talent	L5 – paves way for successors
2	Resourcefulness	Strategic thinker, good under pressure, problem-solver, handles complexity	L5 – Stockdale paradox
3	Straightforward & composure	Steadfast, fact-based, accepts responsibility	L5 – professional will, never blames others
4	Change management	Strategic in facilitating organisational change, overcomes resistance to change	L5 – ambition for company
5	Participative management	Listens, communicates, influences	L5 – Dialogue & debate, not coercion
6	Decisiveness	Prefers quick and approximate over slow and precise	L5 – When change is needed, act
7	Building & fixing relationships	Relational, good negotiator, understands	L5 – Climate where truth is heard
8	Doing whatever it takes	Persistent, takes charge, can stand alone, open to learning	L5 – firm resolve
9	Being a quick study	Quick learner	L5 – Facts are better than dreams
10	Self-awareness	Knows own strengths & weaknesses, willing to improve	L5 – Professional will, personal humility
11	Life-balance	Balances work priorities and personal life	L5 – Professional will, personal humility
12	Confronting problem employees	Decisive but fair	L5 – rigorous, not ruthless
13	Compassion & sensitivity	Shows interest in others, sensitive to needs	L5 – Who you pay, not how you pay.
14	Putting people at ease	Warm, sense of humour	L5 – A culture, not a tyrant
15	Differences matter	Respects differences, culturally aware	L5 – Right people = best asset
16	Career management	Personal development, uses mentors and coaches to manage own career.	L5 – Professional will, personal humility

APPENDIX 4

Summary comparison between Business and Social sectors

Source: Adapted from Collins (2006)

Good-to-Great concepts	Business sector	Social sector
Defining and measuring 'great'	- Widely agreed financial performance metrics - Money both an input and an output - Success defined by returns	- Fewer agreed performance metrics - Money not an input, only an output - Success defined in terms of mission, not returns
Level 5 leadership	- Structures and hierarchies clear - Clear executive powers - Power can substitute leadership	- Structures often ambiguous - Less clear executive powers - True leadership required to get people to follow when they don't have to
First Who – get the right people on the bus	- Difficult to harness idealistic passions - People driven by reward - Easier to get wrong people off the bus	- Easier to tap into ideals and passion - People driven by meaning and nobility of service - Tenures make it difficult to get wrong people off the bus
Confront the brutal facts – living the Stockdale paradox	- Competition forces the facing of facts - Faith that capitalist system works - Belief that best performers will prevail	'Niceness' inhibits candour about facts - System constraints can erode faith in the ability to prevail
Get the 3 circles right	- Economic engine tied directly to profit motive - Only deal in profitable items - Driven by profit and return on investment	- Exist to meet social and human needs - Pricing for profit not possible - Economic engine replaced by resources engine (time, money, brand) - Drivers vary across sectors, but no economic ratio
Culture of discipline	- Profit motive makes it easy to cease a line of activity - Pressures of growth, greed and profit can drive indiscipline	'Do good' desire can drive unwise decisions - Paradoxically, there is less pressure for growth, greed and returns
Flywheel, not doom loop	- Efficient capital markets connect to profit mechanism - Results attract resources, enabling results that attract more resources to fuel greater results – flywheel effect	- No efficient market to reward best results - Flywheel effect still valid for demonstrated success and attractive branding – people support winners
Clock-building, not time-telling	Profit enables sustainability and independence	Funding tied to projects and charisma rather than sustainability motive
Preserve the core, stimulate progress	- Pressures stimulate change and progress - Difficult to preserve core values	Firm mission and core values make it harder to change

Ethics approval



Queensland
Government

Medical Services

Hospital
and Health Service

Enquiries to:
Telephone: (61 7) 1 (61 7)
Facsimile: SSN14/QTDD/1
Our Ref:

Internal Audit

HOSPITAL

Dear

HREC reference number: XXXXX

SSA reference number: XXXXX

Project title: Governance and Leadership in De-centralised Health Services - an early evaluation of the Queensland model.

Thank you for submitting documentation for authorisation of this project. I am pleased to inform you that authorisation has been granted for this study to take place at the following site(s):

Hospital and Health Service- applicable hospitals.

The following conditions apply to this research proposal. These are additional to those conditions imposed by the Human Research Ethics Committee that granted ethical approval.

1. Proposed amendments to the research protocol or conduct of the research which may affect the ethical acceptability of the project are to be submitted to the HREC for review. A copy of the HREC approval/rejection letter must be submitted to the RGO;
2. Proposed amendments to the research protocol or conduct of the research which only affects the ongoing site acceptability of the project, are to be submitted to the research governance officer;
3. Proposed amendments to the research protocol or conduct of the research which may affect both the ongoing ethical acceptability of the project and the site acceptability of the project are to be submitted firstly to the HREC for review and then to the research governance officer after a HREC decision is made.

Yours sincerely

Ethics approval of amendment

Enquiries to: [redacted]
 Telephone: (07) 553 6171
 Facsimile: HREC/14/QTDD/1
 Our Ref:



Queensland
Government

Medicines Services

Hospital
and Health Service

QLD

Dear

HREC Reference number: [number here deleted]
 Project title: Governance and Leadership in Decentralised Health Services an
 early evaluation of the Queensland model.
 Amendment number: XXX
 Amendment Date: 21 May 2014

The above amendment was reviewed by the Chair of the [redacted] Hospital
 and Health Service Human Research Ethics Committee on 21 May 2014.

I am pleased to advise that the amended documents reviewed and approved at the
 meeting were:

Document	Version	Date
Questionnaire: Survey Monkey	2	21 May 2014
Notification of amendment		21 May 2014

The [redacted] Hospital and Health Services HREC is constituted and operates
 in accordance with the National Health and Medical Research Council's *"National
 Statement on Ethical Conduct in Human Research (2007)"*, *NHMRC and Universities
 Australia Australian Code for the Responsible Conduct of Research (2007)* and the
"CPMPIJCH Note for Guidance on Good Clinical Practice".

A copy of this letter must be forwarded to the [redacted] Hospital and Health
 Service Research Governance Officer.

It should be noted that all requirements of the original approval still apply.

Yours sincerely

[redacted]

Interviewee profiles

No.	Date & Time	Profile
1	11 April 2014 11.00am 36 minutes	Note: details of interviewees deleted in this version to ensure confidentiality
2	15 April 2014 2.00pm 39 minutes	
3	16 April 2014 10.00am 40 minutes	
4	17 April 2014 9.00am 43 minutes	

5	22 April 2014 11.00am 47 minutes	
6	24 April 2014 1.00pm 35 minutes	
7	24 April 2014 5.00pm 44 minutes	

8	30 April 2014 2.00pm 44 minutes	
9	30 April 2014 4.00pm 49 minutes	
10	2 May 2014 1.00pm 59 minutes	

Interviewee information and consent form

8th April 2014

Dear _____

Thank you for agreeing to participate in this research project.

The purpose of this note is to provide you with brief details of the project and other information relevant to your participation:

You are advised that:

- The research is on leadership within health sector organisations, and how it may differ from that in non-health sector organisations;
- This research project forms part of the assessment for a Masters degree programme and is not intended for publication;
- The identity of the organisation and participants in the research will not be revealed;
- This project has received the approval of the [organisation] Chief Executive and the Human Research Ethics Committee under reference [reference no]
- The contents of the interview are absolutely confidential, and your identity will not be revealed without your express permission;
- The interview will, with your permission, be digitally recorded and transcribed for analysis purposes;
- You may request a copy of the transcript of the interview;
- Data obtained in the interview will be securely and digitally stored on my personal computer;
- Whilst you have agreed to participate in an interview, you may withdraw at any time;
- You may be asked to participate in a follow-up interview.

To signify your acceptance of the above, please can you sign in the space below.

Should you require any further information regarding this matter, please do not hesitate to contact me.

Regards

Signature

[Researcher signature and contact details]

I agree to proceed with the interview on the basis of the information provided above.

Signed:.....

Date:.....

Name:

Interview Schedule of Questions and Purposes

Question		Purpose
Personal Profile		
1	Have you had any Executive, or Senior Management experience outside of the health sector?	Experience and objectivity when discussing leadership in health.
2	If so, please briefly describe the type of organisation(s).	
3	What is your actual professional background, or qualification?	To determine clinical background, training, and leadership experience. Born or bred?
4	Please briefly describe for me your leadership journey.	
5	Were there any mentors?	
6	Did you receive any formal leadership training?	
Leadership in the Health Sector		
1	In your view, is being a leader in a health services organisation different from any other organisation, commercial or industrial, and if so, in what ways?	Seeking views on the relevance and importance of sector specific skills to leadership. Also seeking an internal definition of leadership for AnHHS.
2	Apart from their professional field, clinical or otherwise, do you think leaders in health should be prepared, or trained in a particular way?	
3	Can you think of any attributes that are required of a health leader, that are not necessarily required in other fields?	
4	In your opinion, what makes a leader effective in the health sector? What kind of things do they do, how do they behave? What does effectiveness look like?	Seeking views on attributes and behaviours that might be peculiar to health, as well as the relevance of clinical skill and experience
5	Do you believe it is necessary to have a clinical background in order to be an effective leader in the health sector?	
AnHHS Senior Leadership		
1	Do you believe AnHHS has leaders that demonstrate the attributes of effective leaders?	Seeking an internal evaluation of the effectiveness of this group.
2	Do you believe that the leadership teams, either the Board or the Senior Management Team, at AnHHS can be described as being effective according to your definition?	
3	In what areas do you think that improvement needs to take place?	
AnHHS Board of Directors		
1	Do you believe that AnHHS Board members demonstrate the attributes of effective leaders?	Seeking an internal evaluation of the effectiveness of this group.
2	Do you believe that the Board at AnHHS can be described as being effective according to your definition?	
3	In what areas do you think that improvement needs to take place?	
Conclusion		
1	Explain transcription – offer copy if interested.	
2	Reiterate confidentiality.	
3	Seek permission for follow-up conversation if required.	
4	Ask if there are any questions.	
5	Thanks.	
Afterwards – make notes about the following:		
1	How the interview went: adequacy of timing, was interviewee talkative, cooperative, nervous etc?	
2	Feelings about the interview: did it give what was expected, open new avenues of interest?	
3	Where it took place, and the setting: quiet, busy, interruptions.	

APPENDIX 10

Level 1 coding of interview transcripts: Frequency of occurrence of key words (high-lighted codes incorporated into questionnaire)

Codes	Type	Qty	Transcript lines in which the codes appeared, by interview ID code									
			T1	T2	T3	T4	T5	T6	T7	T8	T9	T10
accountable	attribute	6		167, 189	246						276	368, 382
adaptable & flexible	attribute	10		322, 350	183, 243		260			356		142, 171, 433, 465
adventurous	attribute	2		122					70			
ambition and drive	attribute	11		66	48, 50	213, 224	39, 46, 96, 546					106, 305
approachable	attribute	5					152	302, 303	257, 445			
brave (takes risks)	attribute	2	208, 320									
challenge and be challenged	attribute	12	288, 341				87, 550, 563		100	380		177, 317, 329, 345, 755
Change	attribute	2	112, 331									
compassionate	attribute	8			249, 437	110					196, 382, 392, 537, 540	
concerned	attribute	1					520					
decisive	attribute	13	358	94, 95, 162, 188, 193, 504		112					512	121, 190, 329, 361
discernment	attribute	2				312			220			attribute
empathy	attribute	8		96	195, 327			355, 372			47	212, 217
ethics	attribute	2		73	160							
focused	attribute	8	235	506	423		292, 454			342, 421		175
forward thinking	attribute	2				541, 552						
friendly	attribute	1				47						
honest (don't hide mistakes)	attribute	5	156, 168	83			572					353
humble	attribute	3					403, 418, 478					
ideals	attribute	1	48									
integrity	attribute	2		73, 82								
intelligent	attribute	8								66, 67, 170, 189, 190		303, 305
kind	attribute		2			545				241		110
objective	attribute	6		391, 393			264	326		80		128
overcomer	attribute					1						89
passion	attribute	5	116				100				96	182, 223
patient	attribute				2						331,	384
persuasive	attribute	2	35				295					
pride	attribute	7	158, 356				302				181, 223, 238, 240	
principles	attribute	1					100					
realistic	attribute	2									368, 576	
responsible	attribute	8		190, 192, 193			67				345, 548	330, 381
self analysis	attribute	2	71, 350									
self awareness & understanding	attribute	17	286, 294, 296, 350	366, 505			107, 135, 153, 313, 314, 392, 572		36, 176			379, 380
self discipline	attribute	7	294, 296				251, 387, 389, 418		170			
strong personality	attribute	2	368, 369									
values	attribute	7	46, 209, 297			307	374, 378, 438					
visionary	attribute	19	297	476	217, 295, 299					470, 475, 565, 571		109, 218, 366, 576, 673, 681, 685, 713, 727,
action	behaviour			1								165
aloof	behaviour		1									180
calm	behaviour			2							237,	241
charismatic	behaviour	1	45									
coach	behaviour	9		226, 239	168, 180				180			450, 623, 629, 758
committed	behaviour	6	138				434		216		181, 183, 197	
communicate	behaviour	24		235, 237	186, 197, 304, 339		106, 109, 117, 431, 478				228, 239, 301, 396, 441, 442	72, 263, 267, 272, 507, 537, 539
competitive	behaviour		2			147						354
confident	behaviour		1									273
consistent	behaviour		1									378
consultative	behaviour	2								296		408
cooperative	behaviour	1			345							

difficult	behaviour	1	319									
diligence	behaviour	13	322		242, 312	133	134, 153	308, 311	51, 76, 86, 217	44, 66	544, 546	
directs & guides	behaviour	14	393		295, 299	373, 405			222	467, 469, 470	65, 91, 272, 275	127
diversity	behaviour	11	369	54	63		51, 57, 75	115			41, 156	195, 465
efficient	behaviour	3	259					173	161			
emotion	behaviour	3	320								199	190
empower	behaviour	1		282								
encourage	behaviour	13	46			60, 62	167		247	215, 216, 218		171, 276, 628, 760, 764
energetic	behaviour	4				545			216	89		108
engaged	behaviour	17	138		339, 401		170, 234, 414, 552, 564	163, 194		484, 486	327, 337, 338, 340,	272
enthusiastic	behaviour	1								89		
example	behaviour	7			218, 313			310	216, 228		186	562
firm	behaviour	1				108						
fun & enjoyment	behaviour	12		79			236, 299, 300			109	188, 191, 192, 224, 357, 582	189
give recognition	behaviour	1	195									
goal oriented	behaviour	3			299		440			356		
helpful	behaviour	2		406			401					
inclusive	behaviour	1					417					
inspire	behaviour	10	288				132, 235, 239		229		566, 568	204, 333, 559
interested	behaviour	7	117				548		150, 474		220, 433, 439	
invest in training	behaviour	5	57, 185, 192, 205									68
journey	behaviour	1	26									
listens	behaviour	14		382	241				219, 395, 400, 406, 412, 430, 474		208, 209, 515	507, 508
loyal	behaviour	3		373							601	158
mentors	behaviour	12	74		178	284	248, 249			219		367, 444, 449, 623, 629, 758
mission focused	behaviour	1		486								
motivation (others & self)	behaviour	18	88, 282	250	327			102	155, 156, 170, 181	289	460, 567	160, 172, 275, 367, 629, 762
nimble	behaviour	6		301						356, 364, 361, 369, 379		
not passive	behaviour	3	392, 399									678
organised	behaviour	3			236							366, 576
ownership	behaviour	1	130									
performance	behaviour	2	190, 210									
persistent	behaviour	1									94	
prepared	behaviour	5	432							93	590, 596, 603	
quality & safety	behaviour	11	263, 287, 332		61				156	481, 483	299, 345, 456, 605	
questioning	behaviour	2	355	366								
relationship	behaviour	30	48, 124, 269, 295, 398		186	379, 471	124, 131, 139, 229, 307		218, 225, 227, 477, 499	208		51, 64, 172, 209, 210, 366, 417, 431, 504, 536, 539
reliable	behaviour	3									184, 185, 246	
resilient	behaviour	3	322								243	551
respect	behaviour	26	269, 270, 271, 280, 473		116, 307, 308	112	132, 140, 312, 383, 415, 418	333, 339	231, 283, 306, 312	297, 299	380, 522	501
right thing	behaviour	2	163, 164									
risk appetite	behaviour	2	393		403							
stamina strong	behaviour	2				136, 142						
(makes hard decisions, deals with 'uncomfortable', has hard & robust conversations)	behaviour	13	198, 325, 326, 357, 368	95, 96, 242, 245, 246		253					367	190
strong (makes hard decisions, deals with 'uncomfortable', has hard & robust conversations)	behaviour	1	357									
succeed	behaviour	1					298					
supportive	behaviour	6	192, 212		321, 375	62				107		
team learning	behaviour	4	78, 298,						227			

(Senge)			451									
teamwork	behaviour	22	283, 461	38, 90, 305, 340	145, 278, 338, 348, 471			114, 253, 374, 478	280, 288, 417	78, 215, 222	652	
trust (and manage trust)	behaviour	9	154, 160, 161, 179, 295	258		132				315, 319		
visible	behaviour	10			275, 336	154, 160	306	439, 442, 468		207, 460		
volunteer	behaviour	1	31									
add value	knowledge	1		458								
advisor	knowledge	3	282	226, 235								
cultural aware	knowledge	4	109		49					436, 540		
environmental awareness	knowledge	4		282		301			375		765	
experience	knowledge	13	65, 375, 378	43, 54, 88, 437	405	56, 255	65, 288		236	43, 55, 70, 116		
frameworks	knowledge	1	264									
knowledge	knowledge	45	59, 391, 468, 473, 477	40, 50, 103, 106	40, 98, 273, 277, 283, 286, 295, 320, 338, 350, 468	161, 164, 377, 545	46, 223	94, 183, 195, 206, 333	37, 57	75, 78, 82, 260	202, 258, 508, 518	54, 62, 308, 599
lack understanding of role	knowledge	1	415									
learning & developing	knowledge	29	365, 370, 403	425, 457	396, 397, 415, 416	473		213, 234				
learns	knowledge		50, 72, 298				59, 64, 150, 247, 258, 261, 563		55	102, 109	594, 595	427, 589
not yet mature	knowledge	1	372									
political awareness	knowledge	11	275, 277, 278			381	200, 344		421	118, 217	201, 415	
resource aware	knowledge	3	262, 287, 332									
succession planning	knowledge	1	189									
training	knowledge	22	391	40, 150	33, 152, 164, 202		225	134	37	106, 129	255, 455, 599, 604	379, 427, 533, 550, 629, 758
understanding	knowledge	39		331, 334, 363	98, 114, 283, 287, 321, 338, 350, 468	265		156, 163, 209	234, 282, 464, 469, 501	93, 250, 295, 334, 421	259, 392, 515, 521	54, 264, 268, 414, 494, 498, 500, 510, 536, 540
complex gender	other	1	28									
group evaluation	other	1	133									
hierarchy	other	1	349									
opportunity	other	1	133									
organisational culture	other	11		66, 74, 87						61, 69, 210, 380	59, 247	289, 290
reputation	other	1	333									
scrutiny	other	2	263, 288									
significance	other	1	174									
deals with complexity	skill	1										
expertise	skill	27								75, 76, 78, 82, 83, 276, 277, 278, 283, 304, 305, 358	240, 507, 532	429, 520, 595
innovative & creative	skill	10		406, 409	36, 56, 60	165, 430, 445, 448						
insight	skill	1	208	78, 281, 283						140, 146, 148	102	762, 763
lack of insight	skill	2	366, 424									
negotiation	skill	2										71, 219
overseer	skill	2		419		392						
quick	skill	2		163								116
skilled	skill	9	369	402, 408	399, 435	429, 508, 520		214				
skilled, capable & competent	skill	27	94, 51, 190, 191, 209	210	98, 219		407, 410	94, 177, 372	232, 235, 301, 391		599	48, 49, 52, 57, 60, 271, 308, 531, 546
strategic thinking	skill	28	393, 438, 446		387, 403, 411, 414	383, 587, 622	443	260		325, 361, 373, 373, 376, 470	215, 423, 427,	262, 341, 576, 694, 713, 719
tasks	skill	1	61									

APPENDIX 11

Level 2 coding of interview transcripts: Frequency of occurrence of themes and concepts

Codes	Qty	No.	Transcript lines in which the codes appeared, by interview ID code									
			T1	T2	T3	T4	T5	T6	T7	T8	T9	T10
Relationship & respect	146	10	46, 48, 91, 126, 154, 161, 163, 165, 176, 179, 182, 269, 272, 275, 280	82, 95, 245, 249, 254, 258,	116, 186, 195, 231, 238, 241, 249, 252, 257, 307, 321, 327, 345, 437, 460,	47, 88, 103, 110, 245, 276, 313, 348, 379, 405, 482,	106, 109, 122, 124, 127, 132, 139, 154, 160, 182, 229, 234, 295, 306, 314, 344, 374, 377, 384, 402, 413, 418, 433, 505,	108, 164, 302, 307, 310, 328, 333, 339,	174, 216, 218, 224, 227, 231, 247, 297, 302, 306, 311, 382, 391, 399, 411, 434, 447, 474,	22, 205, 208, 297, 299, 423, 476,	185, 206, 210, 220, 228, 301, 319, 327, 379, 391, 419, 436, 487, 514, 521, 537, 566, 624	189, 206, 209, 216, 219, 265, 267, 272, 276, 358, 366, 500, 503, 507, 535, 539, 558,
Clinical or not	94	10	45, 52, 90, 155, 208, 269, 275, 277, 353, 38, 94, 147, 184, 190, 194, 464, 473, 476	39, 48, 53, 61, 68, 77, 94, 97, 375, 382, 391, 418,	116, 120, 134, 154, 165, 183, 108, 114, 146, 384, 390,	116, 120, 134, 154, 165, 183, 386, 414, 417, 421,	68, 98, 166, 185, 206, 215, 221, 223, 226, 220, 290, 334, 232, 323,	159, 179, 278, 283, 294, 394, 355,	79, 83, 106, 128, 151, 275, 283, 294, 394, 407,	505, 507, 516, 531,	295, 300, 303, 307, 456, 476, 525, 546, 581,	
Team learning and teamwork	84	9	72, 78, 80, 97, 283, 438, 449, 451, 454, 461,	54, 59, 91, 141, 233, 305, 315, 357, 360,	542, 552, 556,	154, 157, 166, 173, 236, 298, 302, 315, 382, 386, 416, 440, 448, 452, 513,	113, 116, 121, 241, 249, 253, 193, 195, 266, 374, 377, 478, 490	158, 279, 286, 289, 412, 417, 424,	80, 217, 222, 224, 246, 258, 337, 377, 433, 497, 554,	105, 126, 147, 158, 273, 275, 281, 357, 628, 648, 652, 722, 726, 731, 736,		
Know thyself	78	10	50, 66, 110, 209, 286, 295, 309, 322,	83, 117, 212, 216, 238, 242,	159, 162, 183, 286,	100, 274, 301, 308,	108, 135, 150, 251, 258, 269, 291, 313, 388, 393, 403, 409, 546, 550, 562, 574,	102, 383, 410, 361, 56,	41, 67, 86, 89, 91, 96, 109, 167, 171, 180, 190, 250, 320,	183, 187, 202, 576, 582, 591, 596, 605,	133, 142, 145, 167, 171, 203, 223, 353, 373, 415, 419, 430, 444,	
Journey	71	10	26, 36, 57, 108, 419, 428,	41, 46, 57, 63, 78, 109, 122, 198, 219, 425, 457,	31, 43, 50, 63, 81, 89, 135, 176, 292, 417,	68, 82, 99, 157, 163, 229, 271, 392, 492,	31, 46, 61, 75, 86, 100, 112, 150, 159, 372,	81, 96, 119, 213, 234, 242, 409,	58, 73, 77, 100, 117, 136, 143, 197,	46, 139, 161,	45, 82, 386, 392, 599,	
Don't dither, decide	67	9	158, 162, 164, 167, 177, 184, 188, 193, 275, 285, 321, 485, 327, 339,	399, 405, 413, 417, 437, 469, 537,	338, 397, 405, 408, 414, 424, 431,	112, 114, 195, 233, 251, 362, 381, 384, 399, 418, 478,	33, 81, 88, 137, 187, 566, 262,	37, 58, 71, 106, 221,	94, 103, 122, 241,	44, 69, 93, 119, 188, 255, 511,	94, 105, 106, 117, 122, 175, 178, 192, 255, 258, 262, 290, 328, 330, 332, 360, 362, 382, 397,	
The Board	45	10	344, 349, 363, 369, 373, 390,	417, 437, 469, 537,	408, 414, 424, 431,	372, 377, 429, 480, 587,	430,	65, 75, 209, 212, 235, 239, 418,	98, 214	463, 467, 470,	130, 215, 462,	248, 668, 673, 685,
Spot the difference	40	9	84, 280, 283, 288, 290, 295, 302, 312, 324, 219, 223, 227,	330, 342, 349,	218,	336, 340, 598,	320, 324, 330, 341, 351, 356, 371,	151, 155, 163, 183, 197, 326,	245, 253,	310,	464, 466, 468, 475, 748,	
Be bold	33	8	123, 325, 353, 357,	66, 72, 76, 97, 102, 509,	273,	62, 147, 211, 220, 232,	57, 69, 97,	50, 104,		57, 65, 70, 91, 114,	96, 102, 109, 121, 260, 320, 341,	
Which lens	18	8	42, 144, 252, 258, 262,			402,	349,	413, 426,	207,	442, 447, 460, 474,	283,	605, 621, 625,
Contested terrain	11	2	130, 133, 136, 141, 152, 233, 237,		354, 364, 367, 374,							

Quotable quotes from the interviews
(identified by transcript number e.g. T1, T3 etc.)

Themes and concepts	No.	Quotes
Relationships and respect	T2	"...in order to succeed in the private world it seems you have to lie and cheat your way through..."
	T2	"...can't deal with confrontation, so won't hold people to account..."
	T2	"...right people in the right job..."
	T2	"...he trusts me, and that's good."
	T5	"You must have a way of <i>getting</i> information, and a way of communicating what you want to do..."
	T5	"...relationship...connects people and the leader is seen to be credible..."
	T5	"...people have confidence, trust and respect..."
	T5	"I was a walking around person...made the time to walk into every department. Some people saw me almost daily."
	T5	"...leadership is fundamentally connected to relationships..."
	T1	"In healthcare it's all about trust...people must trust you implicitly...to do the right thing."
	T10	"...particularly in finance, the journey towards leadership is usually pretty rocky because no-one prepares you for it. Relationships don't matter, technical skills - that is all you need to get by."
	T10	"To be a sustainable leader you need to be able to [adapt]."
	T10	"...didn't need to nurture staff, particularly in banking and finance."
	T10	"...develop empathy...it's a string to the bow of leadership."
	T10	"...they didn't understand, no-one had talked to them."
	T10	"...the real leadership is what happens in the ward, what the nurse manager is doing to inspire their twelve staff."
Clinical, or not?	T2	"...coming from a medical background, I'm used to a certain level of competence...you don't have to tolerate incompetence so much..."
	T2	"...[not being a clinician was] not a disadvantage because he didn't have any loyalty to any one particular group..." and could therefore be objective.
	T1	"I think it is much harder for people without a clinical background to establish credibility..."
	T3	"...clinicians didn't respect that this person didn't have a clinical background..."
	T3	"...it is my personal belief that you need a clinical background to <i>manage</i> a clinical service."
	T5	"...there are senior clinicians who are leaders [in their field], but there was no organisational leadership."
	T5	"...people think you don't get it because you haven't been in health."
	T5	"I don't believe the fundamentals of leadership are any different anywhere."
	T5	"I think you can lead in healthcare not having been in health at all."
	T5	"He's not a Doctor. He doesn't know what's in a hospital. He doesn't understand."
Know thyself!	T2	"I'd rather he tells me, he should guide and provide...he finds that a little uncomfortable..."
	T5	"...recognise that you're not infallible..."
	T5	"...understand that I was approachable and that I wasn't infallible, and that I wanted to do the best I could."
	T5	"...a good leader knows and is honest with himself."
Team learning and teamwork	T1	"...mobilise your team...to work with you and recruit colleagues...to compliment your gaps."
	T5	"Good leadership is not necessarily knowing about everything in the business, it's having the team around you that knows."
	T1	"I think our leadership is effective...through a difficult period of change, resource constraints and as a team we've kept people safe, provided good service and are building a strong culture."
	T1	"...we're learning together, people from different clinical backgrounds...but I think it's very useful."
	T10	"...it's easier to lead when things are going well than when it is all falling apart."
	T1	"The main weakness that I identify from our executive is the [in]ability to think across the organisation."
	T1	"...on behalf of the whole organisation, not just my bit..."

APPENDIX 12 (continued)

It's a journey	T5	"...journey academically, studying and...went into some leadership roles in administration of hospitals..."
	T5	"...range of roles in leadership from one hospital to a whole group of hospitals..."
	T5	"...I took on bigger and bigger places...and your leadership goes to different levels..."
	T8	"...opportunity is important in determining leadership positions..."
	T10	"...a big challenge in leadership is that it is easier to lead when things are going well than when it is all falling apart."
	T10	"I think there are inherent traits that make you a natural leader that may not come out till later."
Don't dither, DECIDE!!	T2	"I was struck by how people would vacillate so much over a decision."
	T2	"...harvest the information you have at the time, then make a decision, quickly." In health, "...you make the best decision with the information you have, and you get on with it. Otherwise, the patient will expire in front of you."
	T2	"...a lot of indecision...then refer it to another committee..."
	T5	"...the best decision at that time, and for the right reasons."
Spot the difference	T5	"...a multi-professional group...probably no other industry in the world that has so many different groups of professionals in it."
	T10	"The thing about health is the high intelligence of the workforce...[and the difficulty is leading] a bunch of independent, free-thinking, highly educated, highly trained, well-paid staff."
Be bold !	T2	"...give me a chance of running it..."
	T2	"...a smaller company is great for innovation, you just make it up as you go..."
	T1	"...got to find people willing to take risks, to be challenged."
	T5	"...taking on bigger and bigger places...your leadership goes to different levels."
Which lens?	T1	"Effectiveness can be viewed through a number of lenses...community, resources, finance, quality and safety, reputation, so you need a framework."
Contested terrain	T1	"...here, people fight over who owns the patient. Nurses compete with who can do what, and doctors say they can do this and it's hierarchical, a contested domain."
	T1	"...in other sectors... everyone spends their time avoiding owning the client."
	T3	"...expect a group of executives to be competitive but sometimes that can damage..."
	T1	"...here, everybody is fighting for the patient..."
The Board	T2	"They are good people and want to help, but they lack the expertise..."
	T2	"...on a learning curve...and getting better with time..."
	T2	"I don't know if they fully understand strategic planning..."
	T8	"...a permissive Board, rather than being directive."
	T8	"It would be better to get counsel and guidance, than for the Board to interfere where it shouldn't."
	T1	"...still developing and some lack insight as to the role of a Director."
	T1	"...not yet mature, but not dysfunctional either."
	T1	"...should think more broadly than one's own lived experience..."
	T3	"...not a lot of contrary discussion to what the view of the Chair is..."
	T10	"The thing that is missing is Board leadership. I don't see the Board has a vision."
Overall evaluation	T1	"I think our leadership is effective...through a difficult period of change, resource constraints and as a team we've kept people safe, provided good service and are building a strong culture."
	T3	"...they could work more at breaking down those silos and working more together..."
	T7	"...yes, I think it is effective...but there is always room for improvement, in the sense of listening...and being seen."
	T1	"I don't think there is huge depth in our organisation in terms of leadership."
	T8	"...yes, we've now made a profit...we've eliminated waiting lists...we're moving towards achieving national emergency access targets."
	T8	"I believe it's chalk and cheese...and so I think that from a management point of view, we must have been effective."
	T10	"Clearly the leadership team has been effective. Look at the results: surplus...savings... balanced the budget...zero long wait lists...met national targets...doing more activity...maintaining safety and quality..."

Online questionnaire

LEADERSHIP IN THE HEALTH SECTOR

INTRODUCTION and GENERAL INFORMATION

Thank you for your participation!

This survey is being carried out by Student 129113469 as part of the research component of a dissertation on leadership within the health sector.

You are advised that:

- this research is primarily for academic assessment purposes, although the knowledge gained will be highly relevant to the workplace;
- this research is not intended for publication;
- the survey is only being conducted amongst staff of the [name of hospital];
- the organisation will not be identified in the dissertation;
- the survey is being conducted through SurveyMonkey.com and is anonymous;
- the project has received the approval of the [organisation] Chief Executive and the Human Research Ethics Committee (HREC).

Apart from the basic respondent information called for in SECTION A, the survey consists of 3 sections that ask the same questions, but from different perspectives as follows:

- SECTION B - what leadership SHOULD be;
- SECTION C - what leadership currently IS; and
- SECTION D - a SELF EVALUATION.

The survey will remain open until Friday 13th June 2014, and should take you no longer than 10-15 minutes to complete.

If you have any queries relating to questions in this survey, please contact Student 129113469 on Tel No. xxx.

If you have any queries relating to the conduct of this survey, please contact the HREC Coordinator on Tel No. zzz.

Once again, thank you for your participation - it is greatly appreciated.

SECTION A • Personal information

This section requests basic information about you that will greatly assist in the analysis of the survey results.

1. What is your gender?

☐ Female

☐ Male

2. What is your age?

☐ Less than 25

☐ 25 to 34

☐ 35 to 44

☐ 45 to 54

☐ 55 and over

LEADERSHIP IN THE HEALTH SECTOR

3. What is your length of service within the health sector?

- ☐ Less than 3 years
- ☐ 3 to 5 years
- ☐ 6 to 10 years
- ☐ More than 10 years

4. What is your employment status?

- ☐ Fulltime
- ☐ Parttime
- ☐ Contract
- ☐ Casual

Other (please specify)

5. Which of the following best describes your current role?

- ☐ General staff
- ☐ Middle management
- ☐ Senior management
- ☐ Executive management

Other (please specify)

6. Are you a trained clinician?

- ☐ Yes

7. Are you currently employed in a clinical role?

- ☐ Yes

8. Have you ever worked outside the health sector?

- ☐ Yes
- ☐ No

LEADERSHIP IN THE HEALTH SECTOR

9. How many years have you worked outside the health sector?

- ☐ Less than 3 years
- ☐ 3 to 5 years
- ☐ 6 to 10 years
- ☐ More than 10 years

Comments

SECTION B • My view on what leadership SHOULD be

Leadership is a very broad topic and its definition is generally related to context.

It is recognised that within AnHHS, leadership is found at every level of the organisation, and should not be viewed as the domain of senior executives.

This section seeks to identify a definition of effective leadership in AnHHS by asking what leadership SHOULD be.

Please indicate the importance you attach to the following statements.

10. Leaders in AnHHS should be:

	Very important	Important	Moderately important	Of little importance	Not important at all
a. Patient	☐	☐	☐	☐	☐
b. Honest	☐	☐	☐	☐	☐
c. Hardworking	☐	☐	☐	☐	☐
d. Aware of their own limitations	☐	☐	☐	☐	☐
e. Decisive	☐	☐	☐	☐	☐
f. Passionate and driven	☐	☐	☐	☐	☐
g. Intelligent	☐	☐	☐	☐	☐

Comments

LEADERSHIP IN THE HEALTH SECTOR

11. Leaders in AnHHS should:

	Very important	Important	Moderately important	Of little importance	Not important at all
a. Have a deep knowledge and understanding of the health sector	☐	☐	☐	☐	☐
b. Have wide experience in the health sector	☐	☐	☐	☐	☐
c. Demonstrate skill and expertise	☐	☐	☐	☐	☐
d. Be lifelong learners and open to new ideas	☐	☐	☐	☐	☐
e. Be culturally aware and sensitive	☐	☐	☐	☐	☐
f. Be good at organisational politics	☐	☐	☐	☐	☐
g. Be more concerned with day to day issues than the long term future	☐	☐	☐	☐	☐
h. Focus more on creating a work environment that facilitates achievement, rather than the achievement itself	☐	☐	☐	☐	☐

Comments

J

LEADERSHIP IN THE HEALTH SECTOR

12. Leaders within AnHHS should:

	Very important	Important	Moderately important	Of little importance	Not important at all
a. Motivate, encourage and inspire their staff	☐	☐	☐	☐	☐
b. Communicate often, clearly and honestly	☐	☐	☐	☐	☐
c. Invest time and resources in staff training and development	☐	☐	☐	☐	☐
d. Coach, guide and advise staff	☐	☐	☐	☐	☐
e. Take an interest in individual staff members and their families	☐	☐	☐	☐	☐
f. Trust their staff	☐	☐	☐	☐	☐
g. Empower staff with responsibility and authority	☐	☐	☐	☐	☐
h. Monitor and evaluate staff performance	☐	☐	☐	☐	☐
i. Acknowledge good work	☐	☐	☐	☐	☐
j. Confront staff with their bad work	☐	☐	☐	☐	☐
k. Show respect for staff, as individuals and for their skills	☐	☐	☐	☐	☐
l. Be visible and engage with staff	☐	☐	☐	☐	☐
m. Be prepared to hear the views and ideas of staff	☐	☐	☐	☐	☐
n. Be tough, forceful, controlling and prescriptive	☐	☐	☐	☐	☐
o. Foster a team environment	☐	☐	☐	☐	☐
p. Set a good example of values and ethics	☐	☐	☐	☐	☐

Comments

J

SECTION C • My view on what leadership currently IS

This section seeks to evaluate leadership in AnHHS by asking the same questions as in Section B above, but from the perspective of what leadership currently IS.

APPENDIX 13 (continued)

LEADERSHIP IN THE HEALTH SECTOR**15. In general, leaders in AnHHS:**

	Strongly agree	Agree	Neutral	Disagree	Strongly disagree
a. Motivate, encourage and inspire their staff	☐	☐	☐	☐	☐
b. Communicate often, clearly and honestly	☐	☐	☐	☐	☐
c. Invest time and resources in staff training and development	☐	☐	☐	☐	☐
d. Coach, guide and advise staff	☐	☐	☐	☐	☐
e. Take an interest in individual staff members and their families	☐	☐	☐	☐	☐
f. Trust their staff	☐	☐	☐	☐	☐
g. Empower staff with responsibility and authority	☐	☐	☐	☐	☐
h. Monitor and evaluate staff performance	☐	☐	☐	☐	☐
i. Acknowledge good work	☐	☐	☐	☐	☐
Confront staff with their bad work	☐	☐	☐	☐	☐
k. Show respect for staff, as individuals and for their skills	☐	☐	☐	☐	☐
l. Are visible and engage with staff	☐	☐	☐	☐	☐
m. Are prepared to hear the views and ideas of staff	☐	☐	☐	☐	☐
n. Are tough, forceful, controlling and prescriptive	☐	☐	☐	☐	☐
o. Foster a team environment	☐	☐	☐	☐	☐
p. Set a good example of values and ethics	☐	☐	☐	☐	☐

Comments

SECTION D • Self evaluation

This section provides you with an opportunity to EVALUATE YOURSELF within the context of AnHHS, by using the same questions as those used in Sections B and C above. Please be as honest as possible, remembering that this survey is anonymous

APPENDIX 13 (continued)

LEADERSHIP IN THE HEALTH SECTOR**18. In my role at work, as either the leader or a team member, I**

	Strongly agree	Agree	Neutral	Disagree	Strongly disagree
a. Motivate, encourage and inspire	☐	☐	☐	☐	☐
b. Communicate often, clearly and honestly	☐	☐	☐	☐	☐
c. Invest time and resources in training and development	☐	☐	☐	☐	☐
d. Coach, guide and advise others	☐	☐	☐	☐	☐
e. Take an interest in colleagues and their families	☐	☐	☐	☐	☐
f. Trust my staff and colleagues	☐	☐	☐	☐	☐
g. Empower others with responsibility and authority	☐	☐	☐	☐	☐
h. Monitor and evaluate performance	☐	☐	☐	☐	☐
i. Acknowledge good work	☐	☐	☐	☐	☐
j. Confront others with their bad work	☐	☐	☐	☐	☐
k. Show respect for colleagues as individuals, and for their skills	☐	☐	☐	☐	☐
l. Am visible and engage with colleagues	☐	☐	☐	☐	☐
m. Am prepared to hear the views and ideas of colleagues	☐	☐	☐	☐	☐
n. Am tough, forceful, controlling and prescriptive	☐	☐	☐	☐	☐
o. Foster a team environment	☐	☐	☐	☐	☐
p. Set a good example of values and ethics	☐	☐	☐	☐	☐

Comments

J

APPENDIX 14

Required sample size from known populations Source:
Adapted from www.research-advisors.com/tools/SampleSize.htm Accessed
2 July 2014

Population size	Margin of error	
	5.0%	3.5%
10	10	10
20	19	20
30	28	29
50	44	47
75	63	69
100	80	89
150	108	126
200	132	160
250	152	190
300	169	217
400	196	265
500	217	306
600	234	340
700	248	370
800	260	396
1,000	278	440
1,200	291	474
1,500	306	515
2,000	322	563
2,500	333	597
3,500	346	641
5,000	357	678
7,500	365	710
10,000	370	727
25,000	378	760
50,000	381	772
75,000	382	776
100,000	383	778
250,000	384	782
500,000	384	783
1,000,000	384	783
2,500,000	384	784
10,000,000	384	784
100,000,000	384	784
300,000,000	384	784

Text of questionnaire information email – dated 22 April 2014.

Dear colleagues,

I am currently doing some research on leadership, and am trying to determine whether there are any significant points of difference between leadership inside and outside the health sector. The research project is for assessment purposes and forms part of a degree programme that I am undertaking.

The research has received the approval of the HS Chief Executive and the [organisation] Human Research Ethics Committee under reference [reference number here].

You are requested to assist in this research by completing a very short questionnaire. As a colleague, I am very aware of the significant 'survey fatigue' that we all experience and would fully understand if you choose not to respond.

Having said that, I would be very grateful if you do choose to respond, and expect that you would spend around **10 minutes** to complete the questionnaire, which you may do by clicking on the link below.

<https://www.surveymonkey.com/s/LeadershipInTheHealthSector>

You are advised that the survey is facilitated by SurveyMonkey, and is entirely anonymous.

The survey will remain open until **Friday 13th June 2014**.

If you have any queries relating to the conduct of the survey, please contact the HREC Coordinator on [number here].

If you have any queries, or require assistance relating to questions in the survey, please feel free to contact me as per my details below.

Once again, your participation would be greatly appreciated.

With thanks

[Name and job role]

Questionnaire responses schedule

Day	Date	Responses			Notes
		Daily	Total	%	
Friday	23 May	69	69	19.7	
Saturday	24 May	1	70	20.0	Weekend
Sunday	25 May	0	70	20.0	Weekend
Monday	26 May	29	99	28.3	
Tuesday	27 May	4	103	29.4	
Wednesday	28 May	1	104	29.7	
Thursday	29 May	1	105	30.0	
Friday	30 May	2	107	30.6	
Saturday	31 May	1	108	30.8	Weekend
Sunday	1 June	0	108	30.8	Weekend
Monday	2 June	28	136	38.7	Notice of b/fwd end date.
Tuesday	3 June	7	143	40.9	
Wednesday	4 June	1	144	41.1	
Thursday	5 June	11	155	44.3	Reminder of 1 day to go
Friday	6 June	9	164	46.9	Closed

Questionnaire data: ALL respondents

Qualities & Characteristics	Section B (Desired) AnHHS Leaders SHOULD BE			Section C (Actual) AnHHS Leaders ARE		
	Important	Neutral	Not important	Agree	Neutral	Disagree
ATTRIBUTES						
Patient	88.8%	10.0%	1.3%	47.4%	41.7%	10.9%
Honest	99.4%	0.6%	0.0%	57.3%	29.3%	13.4%
Diligent	96.3%	3.8%	0.0%	84.1%	13.4%	2.5%
Self-aware	97.5%	1.9%	0.6%	42.7%	35.0%	22.3%
Decisive	96.2%	3.8%	0.0%	49.0%	31.8%	19.1%
Passionate	93.7%	5.0%	1.3%	74.5%	23.6%	1.9%
Intelligent	85.0%	15.0%	0.0%	63.5%	31.4%	5.1%
Average	93.8%	5.7%	0.4%	59.8%	29.5%	10.8%
SKILLS AND KNOWLEDGE						
Sector knowledge	86.9%	13.1%	0.0%	80.3%	18.5%	1.3%
Sector experience	71.9%	20.6%	7.5%	61.8%	29.3%	8.9%
Skill & expertise	95.6%	4.4%	0.0%	66.2%	28.0%	5.7%
Lifelong learner	98.8%	1.3%	0.0%	43.9%	43.3%	12.7%
Culturally aware	93.8%	5.0%	1.3%	52.6%	36.5%	10.9%
Politically aware	78.1%	18.1%	3.8%	63.7%	28.0%	8.3%
Non-strategic	28.0%	48.4%	23.6%	41.9%	39.4%	18.7%
Strategic thinking	87.5%	12.5%	0.0%	24.2%	38.2%	37.6%
Average	80.1%	15.4%	4.5%	54.3%	32.7%	13.0%
BEHAVIOURS						
Motivate	98.1%	1.9%	0.0%	31.6%	45.8%	22.6%
Communicate	99.4%	0.6%	0.0%	30.1%	46.2%	23.7%
Train & develop	92.4%	7.6%	0.0%	42.3%	32.7%	25.0%
Coach & guide	91.8%	7.0%	1.3%	34.0%	44.2%	21.8%
Interest in staff	56.0%	34.6%	9.4%	32.3%	41.3%	26.5%
Trust	94.3%	5.7%	0.0%	30.8%	46.2%	23.1%
Empower	95.0%	5.0%	0.0%	31.0%	39.4%	29.7%
Evaluate	95.6%	4.4%	0.0%	49.7%	34.2%	16.1%
Praise	97.5%	2.5%	0.0%	40.6%	40.6%	18.7%
Rebuke	93.7%	5.0%	1.3%	35.9%	37.2%	26.9%
Respect	99.4%	0.6%	0.0%	40.6%	40.0%	19.4%
Visible & engage	95.6%	4.4%	0.0%	35.9%	39.1%	25.0%
Listen	98.7%	1.3%	0.0%	39.7%	39.1%	21.2%
Tough & control	17.6%	13.8%	68.6%	44.2%	35.9%	19.9%
Teambuilder	98.7%	1.3%	0.0%	31.8%	44.2%	24.0%
Be example	100.0%	0.0%	0.0%	41.6%	44.2%	14.3%
Average	89.0%	6.0%	5.0%	37.0%	40.6%	22.4%

Questionnaire data: CLINICIAN respondents

Qualities & characteristics	Section B (Desired) AnHHS Leaders SHOULD BE			Section C (Actual) AnHHS Leaders ARE		
	Important	Neutral	Not important	Agree	Neutral	Disagree
ATTRIBUTES						
Patient	85.7%	14.3%	0.0%	61.9%	38.1%	0.0%
Honest	100.0%	0.0%	0.0%	76.2%	14.3%	9.5%
Diligent	100.0%	0.0%	0.0%	85.7%	14.3%	0.0%
Self-aware	100.0%	0.0%	0.0%	61.9%	23.8%	14.3%
Decisive	100.0%	0.0%	0.0%	66.7%	23.8%	9.5%
Passionate	95.2%	0.0%	4.8%	76.2%	23.8%	0.0%
Intelligent	71.4%	28.6%	0.0%	61.9%	33.3%	4.8%
Average	93.2%	6.1%	0.7%	70.1%	24.5%	5.4%
SKILLS AND KNOWLEDGE						
Sector knowledge	81.0%	19.0%	0.0%	71.4%	28.6%	0.0%
Sector experience	71.4%	23.8%	4.8%	52.4%	38.1%	9.5%
Skill & expertise	100.0%	0.0%	0.0%	52.4%	47.6%	0.0%
Lifelong learner	100.0%	0.0%	0.0%	57.1%	33.3%	9.5%
Culturally aware	95.2%	4.8%	0.0%	66.7%	28.6%	4.8%
Politically aware	76.2%	23.8%	0.0%	61.9%	33.3%	4.8%
Non-strategic	19.0%	47.6%	33.3%	57.1%	23.8%	19.0%
Strategic thinking	90.5%	9.5%	0.0%	33.3%	33.3%	33.3%
Average	79.2%	16.1%	4.8%	56.5%	33.3%	10.1%
BEHAVIOURS						
Motivate	95.2%	4.8%	0.0%	52.4%	42.9%	4.8%
Communicate	100.0%	0.0%	0.0%	52.4%	33.3%	14.3%
Train & develop	95.2%	4.8%	0.0%	57.1%	28.6%	14.3%
Coach & guide	85.7%	14.3%	0.0%	57.1%	28.6%	14.3%
Interest in staff	66.7%	23.8%	9.5%	42.9%	38.1%	19.0%
Trust	90.5%	9.5%	0.0%	52.4%	28.6%	19.0%
Empower	100.0%	0.0%	0.0%	52.4%	19.0%	28.6%
Evaluate	100.0%	0.0%	0.0%	81.0%	19.0%	0.0%
Praise	100.0%	0.0%	0.0%	75.0%	20.0%	5.0%
Rebuke	100.0%	0.0%	0.0%	57.1%	28.6%	14.3%
Respect	95.2%	4.8%	0.0%	61.9%	33.3%	4.8%
Visible & engage	100.0%	0.0%	0.0%	38.1%	52.4%	9.5%
Listen	95.2%	4.8%	0.0%	42.9%	47.6%	9.5%
Tough & control	23.8%	9.5%	66.7%	52.4%	33.3%	14.3%
Teambuilder	100.0%	0.0%	0.0%	52.6%	42.1%	5.3%
Be example	100.0%	0.0%	0.0%	55.0%	40.0%	5.0%
Average	90.5%	4.8%	4.8%	55.2%	33.5%	11.4%

APPENDIX 17 (continued)
Questionnaire data: NON-CLINICIAN respondents

Qualities & characteristics	Section B (Desired) AnHHS Leaders SHOULD BE			Section C (Actual) AnHHS Leaders ARE		
	Important	Neutral	Not important	Agree	Neutral	Disagree
ATTRIBUTES						
Patient	90.0%	8.3%	1.7%	46.6%	39.7%	13.8%
Honest	100.0%	0.0%	0.0%	53.4%	27.6%	19.0%
Diligent	96.7%	3.3%	0.0%	79.3%	15.5%	5.2%
Self-aware	96.7%	1.7%	1.7%	31.0%	43.1%	25.9%
Decisive	98.3%	1.7%	0.0%	46.6%	29.3%	24.1%
Passionate	91.7%	8.3%	0.0%	72.4%	25.9%	1.7%
Intelligent	86.7%	13.3%	0.0%	59.6%	31.6%	8.8%
Average	94.3%	5.2%	0.5%	55.6%	30.4%	14.1%
SKILLS AND KNOWLEDGE						
Sector knowledge	83.3%	16.7%	0.0%	84.5%	13.8%	1.7%
Sector experience	70.0%	20.0%	10.0%	67.2%	20.7%	12.1%
Skill & expertise	98.3%	1.7%	0.0%	74.1%	17.2%	8.6%
Lifelong learner	98.3%	1.7%	0.0%	43.1%	41.4%	15.5%
Culturally aware	96.7%	3.3%	0.0%	52.6%	33.3%	14.0%
Politically aware	75.0%	20.0%	5.0%	70.7%	22.4%	6.9%
Non-strategic	32.8%	53.4%	13.8%	35.1%	45.6%	19.3%
Strategic thinking	83.3%	16.7%	0.0%	25.9%	32.8%	41.4%
Average	79.7%	16.7%	3.6%	56.7%	28.4%	14.9%
BEHAVIOURS						
Motivate	98.3%	1.7%	0.0%	23.2%	46.4%	30.4%
Communicate	100.0%	0.0%	0.0%	26.3%	40.4%	33.3%
Train & develop	90.0%	10.0%	0.0%	35.1%	28.1%	36.8%
Coach & guide	93.3%	5.0%	1.7%	33.3%	36.8%	29.8%
Interest in staff	41.7%	46.7%	11.7%	29.8%	40.4%	29.8%
Trust	96.7%	3.3%	0.0%	33.3%	47.4%	19.3%
Empower	91.7%	8.3%	0.0%	33.3%	42.1%	24.6%
Evaluate	90.0%	10.0%	0.0%	42.1%	35.1%	22.8%
Praise	95.0%	5.0%	0.0%	31.6%	42.1%	26.3%
Rebuke	95.0%	3.3%	1.7%	31.6%	33.3%	35.1%
Respect	100.0%	0.0%	0.0%	35.7%	39.3%	25.0%
Visible & engage	88.3%	11.7%	0.0%	31.6%	47.4%	21.1%
Listen	98.3%	1.7%	0.0%	35.1%	36.8%	28.1%
Tough & control	20.0%	16.7%	63.3%	49.1%	24.6%	26.3%
Teambuilder	98.3%	1.7%	0.0%	35.1%	36.8%	28.1%
Be example	100.0%	0.0%	0.0%	45.6%	33.3%	21.1%
Average	87.3%	7.8%	4.9%	34.5%	38.1%	27.4%

Questionnaire data: SELF-EVALUATIONS

Qualities & characteristics	Section B (Desired) AnHHS Leaders SHOULD BE			Section C (Actual) AnHHS Leaders ARE		
	Important	Neutral	Not important	Agree	Neutral	Disagree
ATTRIBUTES						
Patient	88.8%	10.0%	1.3%	80.6%	14.2%	5.2%
Honest	99.4%	0.6%	0.0%	98.7%	1.3%	0.0%
Diligent	96.3%	3.8%	0.0%	95.5%	4.5%	0.0%
Self-aware	97.5%	1.9%	0.6%	78.7%	17.4%	3.9%
Decisive	96.2%	3.8%	0.0%	72.9%	23.9%	3.2%
Passionate	93.7%	5.0%	1.3%	74.8%	24.5%	0.6%
Intelligent	85.0%	15.0%	0.0%	78.7%	19.4%	1.9%
Average	93.8%	5.7%	0.4%	82.9%	15.0%	2.1%
SKILLS AND KNOWLEDGE						
Sector knowledge	86.9%	13.1%	0.0%	72.9%	18.7%	8.4%
Sector experience	71.9%	20.6%	7.5%	69.0%	20.6%	10.3%
Skill & expertise	95.6%	4.4%	0.0%	94.2%	5.8%	0.0%
Lifelong learner	98.8%	1.3%	0.0%	94.8%	5.2%	0.0%
Culturally aware	93.8%	5.0%	1.3%	89.7%	9.7%	0.6%
Politically aware	78.1%	18.1%	3.8%	43.5%	36.4%	20.1%
Non-strategic	28.0%	48.4%	23.6%	39.4%	30.3%	30.3%
Strategic thinking	87.5%	12.5%	0.0%	74.8%	21.9%	3.2%
Average	80.1%	15.4%	4.5%	72.3%	18.6%	9.1%
BEHAVIOURS						
Motivate	98.1%	1.9%	0.0%	90.3%	8.4%	1.3%
Communicate	99.4%	0.6%	0.0%	92.3%	7.1%	0.6%
Train & develop	92.4%	7.6%	0.0%	72.1%	24.7%	3.2%
Coach & guide	91.8%	7.0%	1.3%	92.9%	6.5%	0.6%
Interest in staff	56.0%	34.6%	9.4%	78.7%	16.8%	4.5%
Trust	94.3%	5.7%	0.0%	85.8%	12.3%	1.9%
Empower	95.0%	5.0%	0.0%	87.1%	11.6%	1.3%
Evaluate	95.6%	4.4%	0.0%	81.9%	16.1%	1.9%
Praise	97.5%	2.5%	0.0%	96.8%	3.2%	0.0%
Rebuke	93.7%	5.0%	1.3%	69.0%	22.6%	8.4%
Respect	99.4%	0.6%	0.0%	98.1%	1.9%	0.0%
Visible & engage	95.6%	4.4%	0.0%	96.1%	3.9%	0.0%
Listen	98.7%	1.3%	0.0%	96.8%	3.2%	0.0%
Tough & control	17.6%	13.8%	68.6%	11.8%	26.3%	61.8%
Teambuilder	98.7%	1.3%	0.0%	92.2%	7.8%	0.0%
Be example	100.0%	0.0%	0.0%	97.4%	2.6%	0.0%
Average	89.0%	6.0%	5.0%	83.7%	10.9%	5.4%

**Comparison of importance of qualities of leadership
with their existence within AnHHS**

Quality		Desired	Actual
Attributes	Honesty	99.4%	57.3%
	Self-awareness	97.5%	42.7%
	Diligence	96.3%	84.1%
	Decisiveness	96.2%	49%
	Passion and drive	93.7%	74.5%
	Patience	88.8%	47.4%
	Intelligence	85.0%	63.5%
	Average	93.8%	59.8%
Skills and knowledge	Lifelong learner open to new ideas	98.8%	43.9%
	Skill and expertise	95.6%	66.2%
	Cultural awareness and sensitivity	93.8%	52.6%
	Strategic thinking	87.5%	24.2%
	Sector knowledge and understanding	86.9%	80.3%
	Organisational political awareness	78.1%	63.7%
	Health sector experience	71.9%	61.8%
	Non-strategic thinking	28%	41.9%
	Average	80.1%	54.3%
Behaviours	Lead by example of values and ethics	100%	41.6%
	Respect for staff and their skills	99.4%	40.6%
	Communication, often and clear	99.4%	30.1%
	Team builder and team player	98.7%	31.8%
	Listen to views and ideas of others	98.7%	39.7%
	Motivator, encourager and inspirer	98.1%	31.6%
	Acknowledge and praise good work	97.5%	40.6%
	Visible and engage with staff	95.6%	35.9%
	Monitor and evaluate performance	95.6%	49.7%
	Empower with responsibility and authority	95.0%	31.0%
	Trust staff	94.3%	30.8%
	Confront staff with bad work	93.7%	35.9%
	Invest in staff training and development	92.4%	42.3%
	Coach, guide and advise staff	91.8%	34.0%
	Take interest in staff and families	56.0%	32.3%
	Tough, forceful, controlling and prescriptive	17.6%	44.2%
	Average	89.0%	37.0%